

NE-BC

Nurse Executive Certification

500 Flashcards

Complete Review • All Four Domains • ANCC-Aligned

Leadership

Healthcare Delivery

Quality & Safety

Human Capital

Contents

How to Use These Flashcards

Each page has **2 flashcards** in a side-by-side format: the **question** appears in the colored left cell and the **answer** in the white right cell. Cover the right column to test yourself, then reveal the answer. The cell color identifies the NE-BC domain. Cards are organized by domain so you can target weaker areas with spaced repetition.

Domain	Cards	Coverage
Leadership Transformational & transactional leadership, conflict management, change theory, strategic planning, governance, Magnet	170	
Healthcare Delivery Care coordination, staffing models, population health, telehealth, discharge planning, care transitions	135	
Quality & Safety QI frameworks, patient safety, NDNQI indicators, root cause analysis, accreditation, infection prevention	120	
Human Capital Workforce development, labor law, performance management, succession planning, onboarding, engagement	75	



Leadership

Flashcards 1–170

What are the 4 components of Transformational Leadership?

Idealized Influence (role model), Inspirational Motivation (vision), Intellectual Stimulation (challenge thinking), Individualized Consideration (mentor each person). Transformational leaders inspire followers to exceed self-interest for the good of the organization.

What is Transactional Leadership?

A leadership style based on exchange relationships: followers receive rewards for meeting defined expectations and face consequences for not meeting them. It maintains performance through contingent reinforcement and management by exception.

Define Servant Leadership.

A philosophy where the leader's primary goal is to serve others first. Key behaviors include listening, empathy, healing, awareness, persuasion, conceptualization, foresight, stewardship, commitment to growth, and building community.

What is Situational Leadership (Hersey-Blanchard)?

Leadership style should match the follower's development level. The four styles are Directing (high task/low relationship), Coaching (high task/high relationship), Supporting (low task/high relationship), and Delegating (low task/low relationship).

What are the 4 development levels in Situational Leadership?

D1: Low competence/high commitment (enthusiastic beginner). D2: Low-moderate competence/low commitment (disillusioned learner). D3: Moderate-high competence/variable commitment (capable but cautious). D4: High competence/high commitment (self-reliant achiever).

What is Lewin's 3-stage Change Model?

Unfreeze (create motivation for change by disrupting the status quo), Change (transition to new behaviors, attitudes, or structures), Refreeze (stabilize the new state and make it the standard). Change that skips unfreezing typically fails.

What are Kotter's 8 Steps of Change Management?

1-Create urgency, 2-Build guiding coalition, 3-Form strategic vision, 4-Enlist volunteers, 5-Enable action by removing barriers, 6-Generate short-term wins, 7-Sustain acceleration, 8-Institute change. Each step must be completed before moving to the next.

What is the Vroom-Yetton Decision Model?

A normative model for choosing a leadership decision style based on situational factors. The five styles range from Autocratic (leader decides alone) to Consultative (seeks input) to Collaborative (group decides together). The correct style depends on decision quality requirements and staff acceptance.

What is Roger's Diffusion of Innovation model?

Describes how innovations spread: Innovators (2.5%), Early Adopters (13.5%), Early Majority (34%), Late Majority (34%), Laggards (16%). The chasm between Early Adopters and Early Majority is where many innovations fail to spread.

What are the 5 components of the Magnet Model?

Transformational Leadership, Structural Empowerment, Exemplary Professional Practice, New Knowledge/Innovation/Improvements, and Empirical Quality Results. Magnet designation requires demonstrating excellence across all five components through documented sources of evidence.

What is Shared Governance in nursing?

A model of organizational decision-making that distributes authority, responsibility, and accountability to the nurses closest to the point of care. It operates through councils (Practice, Quality, Education, Research) with defined scope and authority.

Define Strategic Planning in healthcare.

A systematic process for defining an organization's direction and making decisions on allocating resources to pursue that strategy. Steps include environmental scanning (SWOT), mission/vision clarification, goal-setting, tactical planning, and evaluation.

What is a SWOT Analysis?

A framework for strategic planning that identifies internal Strengths and Weaknesses and external Opportunities and Threats. Strengths and weaknesses are within the organization's control; opportunities and threats come from the environment.

What is the Balanced Scorecard?

A strategic management tool with four perspectives: Financial (how do we look to shareholders?), Customer (how do patients see us?), Internal Processes (what must we excel at?), Learning and Growth (how can we improve?). It translates strategy into measurable outcomes.

What is Triple Aim (IHI)?

Three simultaneous goals: improving the patient experience of care (including quality and satisfaction), improving the health of populations, and reducing the per capita cost of healthcare. Added Quadruple Aim includes improving the health and well-being of the care team.

Define Accountability vs. Responsibility in nursing leadership.

Responsibility is the obligation to complete a task. Accountability is answering for the outcomes of decisions and actions, even when tasks are delegated. Nurse executives are accountable for all nursing practice in their organization even when they delegate tasks to managers.

What is the Thomas-Kilmann Conflict Mode Instrument?

A tool that identifies five conflict modes: Competing (win-lose), Collaborating (win-win), Compromising (partial win-win), Avoiding (no engagement), Accommodating (lose to preserve relationship). Each mode is appropriate in specific situations.

What are Tuckman's 5 Stages of Group Development?

Forming (orientation, polite), Storming (conflict, testing), Norming (cohesion, norms established), Performing (high function, task focus), Adjourning (dissolution, reflection). Groups may cycle back to earlier stages when membership or tasks change significantly.

What is Political Advocacy for nurse executives?

Using knowledge, relationships, and evidence to influence policy decisions that affect nursing practice and patient outcomes. The CNO serves as the voice for nursing in the boardroom, with legislators, and in regulatory processes. Advocacy is a professional obligation, not optional.

What is a Nurse Executive's role in Board Governance?

To translate clinical quality and safety data into governance-level reporting, advocate for nursing resources and practice standards, hold board members accountable for quality oversight, and ensure nursing is represented in strategic decisions that affect patient care.

What is Authentic Leadership?

A style based on self-awareness, transparency, ethical behavior, and balanced processing of information. Authentic leaders build trust by aligning actions with stated values. They seek feedback and acknowledge limitations without undermining their authority.

What is Evidence-Based Leadership Practice?

The integration of best research evidence, leadership expertise, and organizational context when making management decisions. It requires the same disciplined approach as evidence-based clinical practice, including systematic evaluation of data and outcome measurement.

Define Professional Practice Environment.

The organizational conditions that support nurses in providing safe, high-quality care.

Key dimensions from the Practice Environment Scale-Nursing Work Index (PES-NWI) include nurse participation in hospital affairs, nursing foundations for quality care, nurse manager ability/leadership/support, staffing and resource adequacy, and collegial nurse-physician relations.

What is the difference between leadership and management?

Leadership is about influencing people toward a vision — it is relational, transformational, and forward-looking. Management is about coordinating resources and processes to achieve defined goals — it is transactional, operational, and present-focused. Effective nurse executives need both capabilities.

What is Position Power vs. Personal Power?

Position power comes from the formal organizational role (legitimate, reward, coercive authority). Personal power comes from individual characteristics (expert power from knowledge, referent power from relationships and respect). Effective nurse executives use both.

What is Complexity Theory in healthcare leadership?

A framework that views healthcare organizations as complex adaptive systems with non-linear relationships, self-organization, and emergent behavior. Rather than top-down control, effective leadership in complex systems requires adaptive, distributed decision-making and learning from feedback.

What is Emotional Intelligence (EI) in nursing leadership?

The ability to recognize, understand, and manage one's own emotions and the emotions of others. Goleman's five components: self-awareness, self-regulation, motivation, empathy, and social skills. EI predicts leadership effectiveness more accurately than IQ in many studies.

What is Ethical Decision-Making Framework for nurse executives?

A structured approach using four principles: Autonomy (respect patient/staff decisions), Beneficence (do good), Non-maleficence (avoid harm), Justice (fair distribution of resources). When principles conflict, the nurse executive must weigh them in context with legal obligations and institutional policy.

What is Distributive Justice in nursing resource allocation?

The fair distribution of benefits and burdens across a population. In nursing, it guides staffing decisions (equitable workload), budget allocation (proportional to need), and scheduling (equitable access to desirable shifts). Justice does not always mean equal — it means proportionate to need.

What is the role of a CNO in a shared governance model?

To define the authority, scope, and accountability of shared governance councils; support councils with resources and information; hold council decisions in the appropriate domain; model accountability; and link nursing governance outcomes to organizational strategy and quality metrics.

What is a Nursing Strategic Plan?

A multi-year roadmap aligning nursing department goals with the organization's strategic priorities. It includes specific, measurable objectives; resource requirements; accountability assignments; timelines; and metrics for evaluating whether goals are achieved. It must be reviewed and updated regularly.

What is Succession Planning?

A deliberate process for identifying and developing high-potential employees to fill key leadership roles when vacancies occur. It involves talent assessment, individual development plans, role rotation, mentoring, and maintaining a pipeline of candidates ready for promotion at any given time.

What is a 360-Degree Feedback Assessment?

A multi-rater performance evaluation collecting input from the leader's supervisor, peers, and direct reports. It provides a comprehensive view of leadership behavior that self-assessment alone cannot capture. The process is most useful when combined with coaching and a structured development plan.

What is Change Fatigue in healthcare organizations?

A state of decreased motivation and engagement caused by exposure to too many simultaneous or sequential change initiatives. Signs include declining compliance with new protocols, increased cynicism about new projects, and slowed implementation timelines. Leaders must sequence changes and celebrate wins to prevent it.

What is an Organizational Culture Assessment?

A systematic examination of the shared values, beliefs, norms, and practices that shape behavior in an organization. Tools include the AHRQ Hospital Survey on Patient Safety Culture (HSOPSC), Competing Values Framework, and structured observation. Culture assessment must precede culture change efforts.

What is a Vision Statement vs. Mission Statement?

Mission describes what an organization does, for whom, and why (its current purpose). Vision describes what the organization aspires to become in the future (its desired future state). Together, they provide direction for strategic planning and communicate organizational identity internally and externally.

What is Nurse Advocacy in the policy context?

Using professional knowledge and voice to influence laws, regulations, and institutional policies that affect patients, nurses, and the healthcare system. The ANA Code of Ethics identifies advocacy as a core nursing obligation. Advocacy occurs at bedside, organizational, and legislative levels.

What is Benchmarking in nursing leadership?

The process of comparing an organization's performance metrics to internal historical data, peer organizations, or national standards to identify gaps and set improvement targets. NDNQI, Vizient, and CMS star ratings are common benchmarking sources for nursing-sensitive indicators.

What is the role of the nurse executive in ethics consultations?

To ensure nurses have direct access to ethics consultations without manager gatekeeping, to support ethical reasoning in clinical teams, to advocate for organizational ethics policies (including conflict-of-interest disclosure), and to model ethical leadership in administrative decisions.

What is a Nurse-Patient Assignment Policy?

An institutional policy defining how patients are assigned to nurses, including criteria for skill mix, acuity weighting, geographical clustering, and charge nurse responsibilities. Must include a process for nurses to raise unsafe assignment concerns (safe harbor provisions) without retaliation.

What is a Root Cause Analysis (RCA)?

A systematic investigation of a sentinel event or serious safety event that identifies contributing factors and underlying organizational causes. RCA follows the principle that most errors stem from system failures rather than individual negligence. Output includes specific corrective actions with accountability and timelines.

What is the difference between a sentinel event and a near-miss?

A sentinel event is an unexpected event that results in death or serious physical or psychological harm. A near-miss (or close call) is an event that could have caused harm but did not, either by chance or through interception. Near-miss reporting is equally important because it reveals system vulnerabilities.

What is an After-Action Review (AAR)?

A structured debrief conducted immediately after an event or initiative to identify what worked, what did not work, and what should be done differently. Originally from military practice, AARs are used in healthcare for simulation debriefs, disaster drills, and quality improvement event reviews.

What is the Competing Values Framework?

An organizational culture model with four quadrants: Clan (flexible, internal focus — collaboration), Adhocracy (flexible, external focus — creativity), Market (controlled, external focus — competition), Hierarchy (controlled, internal focus — control). Healthcare organizations must balance all four for sustained performance.

What is Coalition Building in nursing leadership?

The process of gathering diverse stakeholders (physicians, administrators, finance, patients) around a shared goal to achieve organizational or policy change that no single group could accomplish alone. Coalitions provide political legitimacy and resource diversity for large-scale change initiatives.

What is a Nurse Executive's fiduciary responsibility?

The obligation to manage organizational resources (financial, human, physical) with care, loyalty, and in the organization's best interest. This includes responsible budget management, transparent reporting to the board, and avoiding conflicts of interest that could compromise decision-making.

What is Organizational Learning?

The process by which an organization improves by systematically gathering, analyzing, and applying knowledge from its own experience and external sources. High-reliability organizations build learning into daily operations through structured debriefs, event reviews, and knowledge management systems.

What is Psychological Safety in teams?

Amy Edmondson's concept: the shared belief that team members can speak up, share concerns, admit errors, or ask questions without fear of punishment or humiliation. It is the single strongest predictor of team learning and patient safety reporting in healthcare settings.

What is a nursing float pool?

A group of cross-trained nurses employed by a hospital to fill staffing vacancies across multiple units on short notice. An effective float pool requires: cross-training competency documentation, equitable assignment policies, clear acuity thresholds for float nurse assignments, and recognition of float nurses' contributions.

What are the types of authority a nurse executive holds?

Line authority (direct decision-making power over a chain of subordinates), Staff authority (advisory influence without direct command), Functional authority (control over specific processes across departments, e.g., nursing practice standards), and Personal authority (influence from expertise and relationships).

What is Interprofessional Collaboration?

A practice model in which professionals from different disciplines work together with patients and families toward shared health goals. Effective interprofessional collaboration requires mutual respect, clear role definition, structured communication (SBAR, TeamSTEPPS), and joint accountability for outcomes.

What is a Nursing Professional Practice Model?

A schematic description of a theory, philosophy, and evidence base that frames how nurses practice, collaborate, communicate, and develop professionally. It describes the relationship between nursing care and patient outcomes and guides the nursing department's work across all settings.

What is a PESTEL analysis in nursing strategic planning?

A macro-environmental scanning tool examining Political, Economic, Social, Technological, Environmental, and Legal factors that affect the organization. Used at the front end of strategic planning to identify external forces that will shape the nursing strategy over a 3-5 year horizon.

What is the difference between efficiency and effectiveness in nursing leadership?

Efficiency is doing things right — minimizing resource use to achieve a result (e.g., reducing documentation time per nurse per shift). Effectiveness is doing the right things — achieving the intended outcome (e.g., improving patient outcomes). Leaders must pursue both but must not sacrifice effectiveness for efficiency.

What is a Nursing Dashboard?

A visual display of key performance indicators for nursing quality, workforce, financial, and safety metrics. Dashboards provide leadership with real-time or near-real-time data for decision-making. Effective dashboards combine outcome metrics with process metrics and include trend data and benchmark comparisons.

What is the Magnet Recognition Program pathway?

Hospitals seeking Magnet designation submit an application documenting their adherence to the Magnet Model across five components. ANCC reviewers evaluate written sources of evidence (organizational overview, empirical quality outcomes, structural empowerment, etc.) and conduct an on-site visit before granting designation.

What is an organizational chart and its limitations in nursing?

A visual representation of formal authority and reporting relationships. Limitations: it does not show informal communication channels, does not reflect actual influence, may be outdated, and does not capture the complexity of matrix relationships or interprofessional governance structures common in healthcare.

What is a span of control?

The number of direct reports a manager effectively supervises. Research suggests 8-12 direct reports is the effective range for managers with complex roles requiring developmental oversight. Spans above this limit reduce supervisory quality, while spans below this limit increase management overhead.

What is Appreciative Inquiry (AI)?

A change management approach that focuses on identifying what is working well (the 'positive core') and building on strengths rather than analyzing deficits. AI uses four phases: Discover, Dream, Design, Destiny (or Deploy). It produces higher staff engagement than deficit-focused problem-solving approaches.

What is a Memorandum of Understanding (MOU) in nursing?

A formal but non-binding agreement between two parties (e.g., a hospital and a school of nursing) that describes a mutual intent to collaborate. MOUs define the scope, terms, and responsibilities of the partnership. They are commonly used for nursing education partnerships, research collaborations, and community health programs.

What is an Informal Leader?

An individual who influences others through personal power (expertise, relationships, respect) without holding a formal management title. Informal leaders significantly affect unit culture, change adoption, and staff engagement. Effective nurse executives identify and engage informal leaders as change agents.

What is Participative Leadership?

A style in which the leader involves team members in goal-setting, problem-solving, and decision-making. It increases staff engagement and ownership of outcomes. It is most effective with experienced, motivated staff who have relevant expertise. It may not be appropriate in crisis situations requiring rapid decisions.

What is the importance of nurse executive visibility?

Regular, purposeful presence on nursing units — rounding with staff, observing care delivery, engaging patients — builds trust, enables early identification of safety and morale concerns, reinforces leadership priorities, and signals that frontline practice matters to senior leadership. Absent executives lose credibility and situational awareness.

What is Mission-Driven Leadership?

Leadership grounded in clear organizational purpose that consistently connects daily decisions and long-term strategy to the mission. Mission-driven leaders use the mission to guide resource allocation, resolve conflicts, and inspire staff to persevere through challenges. It provides coherence across the organization.

What is a Performance Improvement Plan (PIP)?

A formal document that defines specific performance expectations that are not being met, the timeline for improvement, the support the organization will provide, and the consequences if improvement does not occur. PIPs must include objective, measurable criteria and be applied consistently to avoid discrimination claims.

What is Executive Presence?

The combination of communication clarity, poise, confidence, and gravitas that enables a leader to command attention and inspire trust in high-stakes settings such as board presentations, legislative testimony, or media interactions. It is a developable skill, not an innate trait.

What are the 5 practices of exemplary leadership (Kouzes and Posner)?

Model the Way (clarify and act on personal values), Inspire a Shared Vision (enlist others in a compelling future), Challenge the Process (seek improvement and take risks), Enable Others to Act (foster collaboration and build trust), Encourage the Heart (recognize contributions and celebrate wins).

What is a Lean Daily Management System (LDMS)?

A structured operational discipline using visual boards, tiered huddles, and leader standard work to drive continuous improvement and surface problems quickly at the front line. LDMS connects frontline staff to middle management to executive leadership through a daily communication chain.

What is Hardwiring in healthcare leadership?

The process of embedding a behavior or process deeply into the organizational culture and system design so it is performed consistently without conscious effort. Hardwired behaviors include pre-procedure safety timeouts, hourly rounding, and hand hygiene — enforced by system design rather than individual memory.

What is an external regulatory survey and how should a CNO prepare?

An external regulatory survey (e.g., Joint Commission, CMS) is a formal assessment of compliance with accreditation standards. CNO preparation includes regular mock surveys, real-time policy compliance monitoring, staff education on standards, documentation integrity reviews, and ensuring leadership availability during the survey.

What is the IHI Breakthrough Series Collaborative model?

A learning system that brings multiple organizations together to work on a shared improvement goal using Plan-Do-Study-Act cycles, shared measurement, and learning through Action Periods and Learning Sessions. The collaborative model accelerates improvement by sharing learning across organizations simultaneously.

What is an operations budget in nursing?

A financial plan for day-to-day expenses including personnel costs (typically 60-70% of nursing budget), supplies, equipment maintenance, and education. Nurse executives must monitor labor cost per unit of service, overtime/agency ratios, and supply cost variances to manage within budget while maintaining quality.

What is a capital budget?

A financial plan for long-term investments in physical assets such as equipment, technology, and building improvements. Capital requests require detailed business cases including ROI projections, clinical need justification, and life-cycle cost analysis. Capital items typically have a useful life over one year and a cost above a defined threshold.

What is Full-Time Equivalent (FTE) in staffing?

One FTE equals 2,080 paid hours per year (40 hours/week $\times$ 52 weeks). It is used to calculate staffing requirements, convert budgeted positions to actual headcount, and compare staffing models across organizations. A 0.6 FTE nurse works 24 hours per week or 1,248 hours per year.

What is Hours Per Patient Day (HPPD)?

A staffing metric that measures the total nursing hours worked (RN + LPN + aide) per patient census day. $HPPD = \text{total nursing hours} \div \text{patient days}$. It is used to set staffing targets, compare units, and monitor labor productivity. Higher HPPD generally correlates with better nurse-sensitive outcomes.

What is a variance report in nursing finance?

A comparison of actual expenditures against the budgeted amount for a defined period. Positive variances (under budget) may reflect staffing vacancies or reduced census. Negative variances (over budget) may reflect overtime, agency use, or increased supply costs. Nurse managers must investigate and explain significant variances.

What is Return on Investment (ROI) in nursing programs?

$$\text{ROI} = (\text{Financial benefit} - \text{Program cost}) \div \text{Program cost} \times 100\%$$

In nursing, ROI calculations for programs like nurse residency, retention bonuses, or safety initiatives must account for avoided turnover costs, reduced agency spend, avoided HAC penalties, and improved reimbursement from quality metrics.

What is the nursing chain of command?

A structured escalation pathway that nurses follow when a patient safety concern is not addressed at the immediate level. Typically:
bedside nurse → charge nurse → nurse manager → house supervisor → CNO/CMO.
Each level has defined authority and accountability. Nurses must document each escalation step.

What is lateral violence in nursing?

Hostile, aggressive, or harmful behavior between nurses in the same organizational level. Examples include verbal abuse, eye-rolling, exclusion, sabotage, and public humiliation. It is not acceptable conduct and is addressed through professional conduct policies, investigation, and progressive discipline.

What is mandatory reporting in nursing leadership?

Legal or accreditation-required reporting of specific events to regulatory bodies. Examples include sentinel events (Joint Commission), healthcare-associated infections (NHSN), abuse/neglect (state agencies), and certain communicable diseases (public health departments). Failure to report can result in accreditation loss or legal liability.

What is an ANCC NE-BC examination?

The ANCC Nurse Executive Board Certified exam assesses competency in four domains: Leadership (34%), Healthcare Delivery (27%), Quality and Safety (24%), and Human Capital Management (14%). The exam has 175 questions (150 scored, 25 unscored), a 3.5-hour time limit, and a passing benchmark of approximately 72%.

What is the difference between a policy and a procedure?

A policy is a formal statement of an organization's position, values, or requirements on a specific topic (what must be done and why). A procedure is the step-by-step description of how to perform a specific task or implement a policy (how it is done). Both require regular review, approval, and version control.

What is telehealth nursing leadership?

Oversight of nursing practice delivered through remote communication technology. Nurse executive responsibilities include ensuring state licensure compliance across jurisdictions, validating telehealth-specific nursing competencies, designing safe triage protocols, and measuring telehealth-specific patient outcome metrics.

What is an organization's accreditation status and why does it matter?

Accreditation (e.g., Joint Commission, ANCC Magnet, DNV) certifies that an organization meets defined quality and safety standards. It affects CMS certification, payer contracts, professional credibility, and nursing recruitment. Loss of accreditation can jeopardize Medicare/Medicaid reimbursement and trigger regulatory oversight.

What is workforce planning in nursing?

A proactive process for ensuring the right number of nurses with the right skills are available at the right time. It includes demand forecasting (patient volume projections), supply analysis (current workforce + pipeline), gap identification, and strategies to close gaps through recruitment, development, and retention.

What is Crisis Standards of Care?

A framework for reallocating healthcare resources ethically and systematically when demand exceeds normal capacity (e.g., pandemic, mass casualty event). Standards include explicit triage criteria, communication protocols, documentation requirements, and liability protections for clinicians operating under declared emergency conditions.

What is Stakeholder Analysis in nursing leadership?

A process for identifying all individuals and groups affected by or with influence over a decision or change initiative. Each stakeholder is assessed for their level of interest, influence, and support/opposition. Stakeholder analysis informs engagement strategies, communication planning, and coalition-building for change initiatives.

What is a Nursing Compact License?

The Nurse Licensure Compact (NLC) allows RNs and LPNs/VNs to hold one multistate license with the privilege to practice in other compact states. The APRN Compact extends similar multistate privileges to advanced practice nurses. Nurses must practice under the laws of the state where the patient is located.

What is a Nurse Practice Act?

State law that defines the legal scope of nursing practice, standards for licensure, grounds for disciplinary action, and the authority of the state board of nursing. Nurse executives must know their state's Nurse Practice Act and ensure all nursing practice within their organization complies with its requirements.

What is a nursing governance model?

The formal and informal structures through which nurses participate in decisions that affect their practice. Models include shared governance councils, professional practice councils, unit-based councils, and system-level councils. Effective governance models define scope of authority, decision-making processes, and accountability structures.

What is a Nurse Executive's role in population health?

Nurse executives align nursing practice models, staffing, and community partnerships to address the health needs of defined populations. This requires data analysis of patient demographics, chronic disease burden, and care gaps. Community benefit programs and accountable care arrangements are primary levers for population-level nursing impact.

What is an organizational mission statement?

A concise statement of the organization's purpose — what it does, for whom, and why it matters. The mission guides resource allocation and decision-making. In nursing, the mission is operationalized through the nursing vision, professional practice model, and annual strategic priorities. Mission drift is a risk when financial pressures dominate decision-making.

**What is a Nurse Executive
Evidence-Based Decision Framework?**

A structured process for applying research evidence, organizational data, stakeholder values, and leadership judgment to management decisions. It mirrors clinical EBP: ask a management question, search for evidence, critically appraise, apply to context, evaluate outcomes. Decisions without evidence review risk repeating known failed approaches.

**What is a nursing strategic
communication plan?**

A plan specifying how nursing leadership will communicate key messages to different audiences (staff, physicians, board, community) across multiple channels (huddles, newsletters, town halls, dashboards) throughout the strategic planning cycle. Without a communication plan, strategy execution fails at the staff adoption level.

What is a Nurse Executive's responsibility for regulatory compliance?

The CNO is accountable for ensuring all nursing practice within the organization complies with federal and state regulations, accreditation standards, scope of practice laws, and institutional policy. This requires ongoing regulatory surveillance, policy review cycles, staff education, and proactive deficiency identification before surveys.

What is Interdisciplinary Team (IDT) leadership?

Leading a team of professionals from multiple disciplines toward a shared patient care goal. Effective IDT leadership requires: clarifying roles, establishing psychological safety for dissenting views, structuring communication (structured rounding, SBAR), resolving scope-of-practice tensions, and measuring team performance against shared outcome metrics.

What is a Board Quality Committee?

A standing governance committee of the hospital board responsible for oversight of quality, safety, and patient experience metrics. The CNO typically presents nursing quality data and safety event reports to the board quality committee. Board-level quality oversight is a CMS Conditions of Participation and TJC accreditation requirement.

What is a Nurse Executive's role in disaster preparedness?

Ensuring the nursing department has a functional all-hazards plan, sufficient trained staff for mass casualty events, participation in hospital incident command system (HICS), documented cross-training for disaster response roles, supply stockpiling, and documented drills with identified lessons learned for improvement.

**What is transformational culture change
in nursing?**

A deliberate, sustained process of shifting an organization's shared values, behaviors, and norms from a current state to a desired state.

Culture change requires persistent, visible leadership commitment, structural changes that reinforce new expectations, data feedback loops, celebrating early wins, and patience — culture change takes 3-5 years minimum.

**What is the purpose of a nursing annual
report?**

A formal document reporting on the nursing department's performance, accomplishments, quality outcomes, workforce data, and initiatives over the prior year. It demonstrates nursing's value to the organization, supports Magnet documentation, communicates nursing identity to staff and stakeholders, and benchmarks year-over-year progress.

What is a nurse executive's role in informed consent?

The CNO establishes policy ensuring nurses verify (not obtain) informed consent before procedures, recognize when patients require re-consent due to changed conditions or updated information, and escalate concerns about inadequate consent to physicians and the ethics committee. Nurses are responsible for flagging consent deficiencies, not for obtaining medical consent.

What is a Nursing Budget Variance Analysis?

A systematic review of the difference between budgeted and actual expenses in the nursing department. Key variances to analyze: labor (overtime, agency, registry vs. budget), supply costs (unit-level utilization), and census-driven volume variances. Variance explanations must be documented, and corrective action plans developed for negative variances exceeding defined thresholds.

What is a Nursing Department Organizational Restructure?

A formal redesign of the nursing department's reporting relationships, scope of roles, and governance structure to better align with strategic priorities, patient population changes, or financial imperatives.

Restructuring requires systematic stakeholder engagement, communication planning, change management discipline, and impact assessment on patient care continuity.

What is the difference between a nurse executive and a nurse manager?

A nurse executive (CNO, VP of Nursing, Director of Nursing) holds system-level accountability for all nursing practice, workforce, quality, and financial performance across the organization. A nurse manager holds unit-level accountability for a specific department. The nurse executive sets strategy and governance; the nurse manager implements operations.

What is Appreciative Inquiry's 4D Cycle?

Discover (identify what works best — strengths and high points), Dream (envision what could be), Design (co-create the desired future with stakeholders), Destiny (implement and sustain the new design). AI's strength is its inclusivity and positive framing, which reduce resistance to change compared to problem-focused models.

What is a shared mental model in team performance?

A shared understanding among team members of the team's purpose, each member's role, how decisions are made, and what constitutes effective performance. Teams with strong shared mental models coordinate better under pressure without explicit communication. TeamSTEPPS techniques build shared mental models through structured briefings and debriefings.

What is a formal mentoring program for nurses?

A structured program pairing less experienced nurses with senior nurses or leaders for career development. Effective programs include: matching criteria, defined goals and meeting frequency, training for mentors, program evaluation metrics, and institutional support (protected time, recognition). Mentoring improves retention and leadership pipeline development.

What is nursing documentation compliance?

The degree to which nurses complete required documentation (assessments, care plans, interventions, evaluations) accurately, completely, and within defined timeframes. Compliance is monitored through EHR audit reports. Nurse executives must ensure documentation standards are evidence-based, not burdensome to the point of distracting from patient care.

What is an Environmental Scan in nursing strategic planning?

A systematic review of the internal and external factors that will affect the nursing department's ability to achieve its goals over the planning horizon. Internal factors: workforce supply, quality metrics, financial resources, technology. External factors: regulatory environment, payer mix, labor market, competitor landscape, technology trends.

What is a nursing float policy?

An organizational policy specifying when, how, and to which units nurses may be floated, the competency requirements for floating, how float assignments are prioritized and rotated equitably, and the process for nurses to raise safe competency concerns before accepting a float assignment. Float policies must be consistently applied without coercion.

What is Lean Leadership in nursing?

A management philosophy in which leaders actively develop staff problem-solving skills, spend time at the frontline (gemba walks), coach rather than direct, and continuously improve standard work. Lean leadership contrasts with directive command-and-control management. It is the sustainable engine of Lean improvement after initial kaizen events.

What is a nurse executive's role in patient complaints?

Establishing a formal grievance process (required by CMS CoPs) that ensures timely, thorough, documented responses to patient complaints. The CNO reviews aggregate complaint patterns to identify systemic quality issues. Individual complaints are investigated and resolved at the appropriate level. Complaint data is trended for performance improvement.

What is Nurse Staffing Sensitivity Analysis?

A financial modeling technique that estimates the impact of different staffing levels (e.g., varying HPPD targets, agency vs. permanent ratios) on quality outcomes and total cost. Sensitivity analysis informs business cases for safe staffing investments by quantifying the ROI of better staffing versus the cost of adverse events attributed to understaffing.

What is a nurse executive's obligation regarding whistleblower protections?

Federal law (False Claims Act, OSHA Section 11(c)) and state laws protect nurses who report safety violations, fraud, or regulatory non-compliance from retaliation. Nurse executives must ensure that retaliation policies are in place and enforced, that reporting channels are accessible without management gatekeeping, and that whistleblower incidents are investigated independently.

What is a delegation framework in nursing?

NCSBN's five rights of delegation: right task, right circumstance, right person, right direction/communication, right supervision. Delegation does not transfer accountability — the delegating nurse remains accountable for the outcome. Nurse executives must ensure organizational policies align with the state Nurse Practice Act on delegation scope.

What is Executive Coaching for nurse executives?

A structured development relationship between a nurse executive and a professional coach focused on expanding the leader's effectiveness, building self-awareness, and achieving specific leadership goals. Executive coaching is distinct from mentoring (role model relationship) and performance counseling (remedial focus). Evidence supports coaching for leadership development.

**What is a Governance vs. Management
role distinction?**

Governance: the board's responsibility to provide oversight, set strategic direction, and hold management accountable.

Management: the executive team's responsibility to implement strategy and run operations. The CNO must prepare accurate, transparent reports to the board without filtering information that might reflect poorly on operations.

**What is an organizational change
readiness assessment?**

A systematic evaluation of the organization's capacity to successfully implement a specific change. Dimensions assessed: leadership alignment, staff motivation, resource availability, past change track record, communication infrastructure, and cultural factors. Readiness gaps are addressed before launch to improve change success rates.

What is a Nurse Executive's role in clinical research?

Supporting the infrastructure and culture for nursing research, ensuring IRB compliance for nursing studies, facilitating collaboration between nursing staff and academic partners, integrating research findings into evidence-based practice, and advocating for resources (librarian access, release time) that enable nurses to engage in research and EBP.

What is Human-Centered Design in nursing?

An approach to designing care processes, environments, and technologies by deeply understanding the needs, experiences, and constraints of the people who use them — nurses, patients, and families. Methods include ethnographic observation, journey mapping, and rapid prototyping. Human-centered design produces more usable and effective solutions.

What is a Clinical Nurse Specialist (CNS) role in leadership?

The CNS is an advanced practice nurse with expertise in a specialty area who functions as an expert clinician, educator, researcher, and consultant within an organization. CNSs report to the CNO in most structures. They lead EBP projects, mentor staff, improve outcomes in their specialty, and bridge clinical practice and nursing administration.

What is Collective Impact in population health?

A framework for cross-sector collaboration (health systems, schools, housing, transportation, community organizations) around a shared agenda to solve complex social problems affecting population health. Collective impact requires: common agenda, shared measurement system, mutually reinforcing activities, continuous communication, and backbone support organization.

What is a nursing peer support network?

An informal or formal network within the nursing department providing emotional and professional support among nurses facing similar challenges. Peer support networks improve retention, reduce burnout, build psychological safety, and create pathways for informal mentoring. Nurse executives should create structural support for peer networks without over-formalizing them.

What is a pay equity audit?

A systematic analysis of compensation data by gender, race, experience level, and role to identify unexplained pay gaps. Federal law (Equal Pay Act, Title VII) prohibits discriminatory compensation. Nurse executives in HR partnerships should request periodic pay equity audits and develop remediation plans for identified gaps.

What is Servant Leadership applied to nurse managers?

Nurse managers who practice servant leadership prioritize staff needs over personal recognition, remove barriers that prevent nurses from doing their best work, develop staff capabilities through coaching, and build a culture of mutual respect and accountability. Servant leadership is correlated with lower turnover and higher nurse engagement scores.

What is a nurse executive's responsibility for Magnet documentation?

Preparing the organization's Magnet application by compiling sources of evidence (SOEs) for all five Magnet model components, ensuring empirical quality outcomes meet benchmark requirements, coordinating nursing governance documentation, and preparing nursing staff to articulate their practice during the Magnet appraiser site visit.

What is Nurse Executive Leadership Development Program?

A formal curriculum providing current and aspiring nurse executives with competencies in finance, strategy, operations, human capital, quality, and governance. Programs include: AONE/ACHE leadership courses, executive education at business schools, mentoring with experienced CNOs, and rotational assignments in non-nursing areas such as finance and operations.

What is a Nursing Grand Rounds?

A formal educational forum in which nursing staff, educators, or researchers present complex clinical cases, quality improvement findings, or new evidence to nursing colleagues. Grand rounds build a culture of intellectual curiosity, EBP, and inter-unit learning. Nurse executives support grand rounds by providing protected time and recognizing presenters.

What is a nursing competency gap analysis?

A systematic comparison of the knowledge, skills, and behaviors required for current and future nursing roles against the demonstrated competencies of the existing workforce. Gap analysis results inform: orientation redesign, continuing education priorities, hiring specifications, and career development planning for identified high-potential staff.

What is a nursing department town hall?

A large-group open forum in which nursing leadership communicates strategic priorities, quality performance, organizational changes, and responds to staff questions. Town halls build transparency, reduce rumor-driven anxiety, and reinforce the CNO's visibility and accessibility. They should occur at minimum quarterly and include all shifts.

What is a hospital-based nurse education committee?

A multidisciplinary committee including nursing education staff, clinical staff, nurse managers, and the CNO that oversees the design, delivery, and evaluation of nursing education programs. The committee establishes educational priorities aligned with quality metrics, regulatory requirements, and strategic initiatives, and ensures that education resources are efficiently deployed.

What is Kotter's sense of urgency (Step 1)?

Creating a genuine, compelling sense of urgency about the need for change — not manufactured panic, but honest evidence that the status quo is untenable. Kotter argues that 75% of leadership must believe change is necessary before a transformation initiative will succeed. Without urgency, complacency kills change before it begins.

What is a Nursing Executive Fellowship?

A structured experiential learning program providing nurses with intentional exposure to executive-level responsibilities across nursing and non-nursing domains. Fellowships typically span 12-24 months and combine operational projects, executive coaching, didactic coursework, and mentoring with senior leaders. They accelerate advancement into CNO-track roles.

What is a non-punitive safety reporting system?

A reporting system in which nurses and all staff can report safety events, near-misses, and unsafe conditions without fear of disciplinary action, personal liability, or retaliation. Voluntary reporting requires demonstrated organizational response: staff must see that reports lead to system improvements, not to investigations of individuals.

What is a Nurse Executive's use of balanced scorecard data?

The balanced scorecard integrates financial, customer (patient experience), internal process (quality/safety), and learning/growth (workforce) metrics into a strategic monitoring tool. The CNO uses balanced scorecard data at governance meetings to show that nursing performance is comprehensive — not just financially or clinically focused.

What is an interprofessional governance council?

A formal committee including physicians, nurses, pharmacists, therapists, and other disciplines that makes shared governance decisions about clinical quality standards, practice guidelines, and patient care policies. Effective interprofessional governance requires clear scope of authority, representation, decision rules, and accountability to the medical staff and nursing governance structures.

What is an Equity, Diversity, and Inclusion (EDI) strategy in nursing?

A systematic approach to building a nursing workforce that reflects the diversity of the community served, ensuring equitable access to career advancement, and creating an inclusive culture where all nurses can contribute fully. EDI strategy requires data-driven analysis of representation gaps, intentional policy interventions, and leadership accountability for EDI outcomes.

What is the role of a nurse executive during a CMS survey?

To ensure leadership availability throughout the survey, respond to surveyor questions about nursing systems, policy, and staffing with accurate and transparent information, facilitate surveyor access to staff and records, and document all surveyor findings for immediate corrective action planning regardless of whether a deficiency citation is expected.

What is the IHI CUSP (Comprehensive Unit-based Safety Program)?

A methodology for building a safety culture through a structured combination of: educating staff on science of safety, identifying and learning from defects, understanding the science of safety, engaging senior leadership in partnership, and implementing evidence-based toolkits. CUSP was the foundation of the original Michigan CLABSI reduction collaborative.

What is a Nurse Executive's role in magnet re-designation?

After initial designation, Magnet hospitals must seek re-designation every four years by submitting updated sources of evidence demonstrating sustained and improved performance across all five Magnet model components. The CNO leads this process, ensuring continuous documentation of empirical outcomes, governance structure, and professional practice model evolution.

What is Patient Classification System (PCS)?

A tool for categorizing patients by acuity level and care complexity to allocate nursing resources appropriately. PCS data is used to calculate required staffing hours per shift based on patient care needs rather than census alone. Effective PCS implementation reduces both under- and over-staffing by matching resources to actual patient need.

What is a Nurse Executive's financial stewardship role?

Ensuring nursing resources are spent efficiently and in alignment with the organization's mission. This includes: managing the nursing operating and capital budgets, achieving revenue capture for nursing services, demonstrating ROI for nursing programs, and preventing waste without compromising care quality or workforce stability.

What is a nurse-led quality improvement council?

A standing governance body composed of front-line nurses and nursing leaders that reviews quality data, identifies improvement priorities, sponsors PDSA projects, and tracks progress toward nursing quality goals. Nurse-led QI councils are a core element of shared governance and demonstrate professional nursing accountability for outcomes.

What is a delegation register?

A formal record tracking which tasks have been delegated to which staff members, the scope of authority granted, the supervision requirements, and the outcome of each delegation. Delegation registers support accountability, provide evidence for performance management, and ensure that delegated tasks are followed up systematically.

What is a nursing stakeholder map?

A visual representation of all individuals and groups who affect or are affected by nursing department decisions. Stakeholders are mapped by their level of interest and influence. The map guides communication strategy: high interest/high influence stakeholders need intensive engagement; low interest/low influence stakeholders need basic information only.

What is a Nurse Executive's role in quality accreditation surveys?

Preparing the organization for accreditation by maintaining continuous survey readiness, leading mock tracers, ensuring policy currency, validating staff competency in standards knowledge, and modeling the culture of compliance as a patient safety obligation rather than a regulatory exercise.

What is Inter-rater Reliability in performance evaluations?

The consistency of ratings across different evaluators for the same employee or behavioral performance standard. Low inter-rater reliability indicates that ratings are subjective and inconsistently applied, creating legal risk. Nurse executives improve inter-rater reliability through calibration meetings, evaluation training, and defined behavioral anchors.

What is Nurse Executive compensation benchmarking?

Comparing the CNO's and nursing leadership team's compensation to market data from peer organizations. Nurse executives use market benchmarking to advocate for competitive nursing leadership salaries, prevent talent loss to competitor organizations, and ensure compensation aligns with scope of responsibility.

What is a nurse executive's role in HCAHPS performance?

Designing and sustaining nursing practice standards that drive HCAHPS domain performance: nurse communication, responsiveness, pain management, medication education, and overall care rating. HCAHPS performance is linked to VBP payment adjustments. The CNO establishes accountability for HCAHPS scores at unit and department levels.

What is a formal nursing job description?

A document specifying the purpose, essential functions, required qualifications (education, licensure, experience), competencies, physical demands, and reporting relationships for a nursing position. Job descriptions must reflect actual work, be updated when roles change, and align with applicable regulatory scopes of practice. They are the legal foundation for performance management.

What is the Donabedian framework used for in nursing leadership?

Nurse executives use the Donabedian model to evaluate nursing quality comprehensively: Structure (staffing ratios, nurse education levels, equipment), Process (bundle compliance, documentation completeness, care coordination actions), Outcome (HAI rates, falls, readmissions, patient satisfaction). Optimizing structure and process drives better outcomes.

What is a nurse executive leadership philosophy statement?

A personal declaration of the leader's core values, beliefs about the role of nursing, and the leadership principles that guide their decision-making. Leadership philosophy statements build transparency with staff, provide a foundation for difficult decisions, and guide the CNO's self-development priorities. They should be reviewed and updated as experience grows.

What is the Iowa Model of Evidence-Based Practice?

A decision-making model guiding EBP implementation. Steps: problem/knowledge trigger, form a team, assemble evidence, critique and synthesize, sufficient evidence to change practice?, pilot change, evaluate, disseminate. It has been updated to include consideration of external sources of evidence (national guidelines, benchmarks) as triggers.

What is Rapid Cycle Improvement?

An improvement approach based on frequent small PDSA cycles completed in weeks rather than months-long projects. Rapid cycles allow teams to test changes quickly, learn from small-scale failures, and build toward reliable improvement. The IHI Collaborative model pioneered rapid cycle improvement in healthcare quality.

What is a Nurse Staffing Committee (NSC)?

A committee — required in some states by law — that includes direct care nurses and management to develop, review, and update the hospital staffing plan. The NSC evaluates staffing adequacy data, reviews complaints related to staffing, and recommends staffing plan changes. Nurse participation is required by law in states like California and Washington.

What is a nursing-sensitive financial metric?

A financial outcome directly influenced by nursing practice. Examples include: cost per patient day (nurse labor component), agency/overtime cost percentage, cost of HAC penalties avoided through nursing quality, nurse turnover replacement cost, and HAC-related non-payment revenue impact. Nurse executives must track these to quantify nursing's financial contribution.

**What is a Chief Nursing Officer (CNO)
vs. Chief Nurse Executive (CNE)?**

CNO: a common title for the senior-most nursing leader at a hospital or health system.
CNE: a less common alternative title for the same role, sometimes used in complex multi-entity health systems to distinguish a system-level executive from facility-level CNOs. Both roles hold system-wide accountability for nursing practice, quality, and workforce.

**What is a Nursing Clinical Expert
Group?**

A specialty-focused group of clinical nursing experts who advise leadership on practice standards, evidence-based protocols, and workforce requirements for a specific patient population or practice area (e.g., oncology, pediatrics, critical care). Expert groups provide clinical legitimacy for nursing policy decisions.

What is a Performance Dashboard Review Process?

A regular structured review of key performance indicators by nursing leadership, including trending analysis, benchmark comparisons, identification of special cause variation, and decision-making about corrective actions. Effective dashboard reviews move from data observation to root cause investigation to actionable improvement plans with assigned accountability.

What is a Nursing Communication Cascade?

A structured information-sharing process that ensures key messages from senior leadership reach front-line staff in a timely and consistent manner. A typical cascade: CNO briefs directors → directors brief managers → managers brief charge nurses → charge nurses brief staff at huddle. Communication logs document that each cascade step occurred.

What is a Nursing Capital Equipment Request Process?

A formal process for nurse managers and nursing staff to submit evidence-based requests for new or replacement capital equipment. Effective processes include: standardized business case template (need, evidence, cost, alternatives, ROI), nursing committee review, prioritization criteria, budget cycle alignment, and feedback to submitters on request disposition.

What is a nurse executive's role in reducing nursing administrative burden?

Identifying and eliminating documentation, reporting, and meeting requirements that consume nursing time without adding clinical value. Administrative burden is a major driver of nurse dissatisfaction and burnout. The CNO should conduct regular administrative task audits with front-line nurses and advocate for EHR and workflow optimizations that return time to patient care.

What is a Structural Empowerment source of evidence (Magnet)?

For Magnet re-designation, structural empowerment is demonstrated through evidence of nurse access to professional development, formal and informal power structures that support nursing practice, organizational support for community involvement, and recognition programs. Examples of SOEs: clinical ladder data, professional certification rates, conference attendance data, and community service partnerships.

What is a nurse executive's role in telehealth policy?

Establishing nursing-specific telehealth practice standards, ensuring telehealth nurses meet state licensure requirements for multi-state practice, defining competency validation for telehealth roles, designing patient safety protocols for virtual triage, and monitoring telehealth-specific patient outcome metrics as part of the nursing quality program.

What is a Safety Culture Maturity Model?

A framework for assessing an organization's stage of safety culture development: Pathological (who cares as long as we don't get caught), Reactive (safety is important — we do a lot every time we have an accident), Calculative (we have systems in place to manage all hazards), Proactive (we work on the problems we still find), Generative (safety is how we do business here).

What is an After-Hours Administrative Coverage Plan?

A formal plan ensuring nursing leadership coverage is available outside business hours for urgent decisions, safety escalations, and regulatory requirements. Options include: on-call administrator rotation, designated house supervisor with defined decision authority, and direct CNO contact for specified sentinel-level events. The plan must be documented and staff must know how to activate it.

What is the role of nursing in hospital strategic planning?

Nursing brings clinical expertise, quality data, workforce insight, and patient/community perspective to the strategic planning table.

The CNO ensures nursing goals are integrated into the organizational strategic plan rather than treated as a separate nursing silo. Nursing strategy must directly support the organization's mission and population health priorities.

What is a Key Performance Indicator (KPI) in nursing?

A measurable value that tracks progress toward a specific nursing strategic or operational goal. Effective KPIs are: specific, measurable, achievable, relevant, and time-bound (SMART). KPIs should be linked to organizational strategy, benchmarked against external peers, reviewed at consistent intervals, and assigned to accountable owners.

What is Nursing Peer Mentoring?

A developmental relationship between two nurses at a similar career stage who provide mutual support, accountability, and learning. Peer mentoring differs from senior-to-junior mentoring in its reciprocal nature. Programs can be structured (matched pairs, defined meetings) or organic (encouraged through culture). Peer mentoring builds psychological safety and retention.

What is a Nurse Leader Rounding program?

A structured practice in which nurse managers or senior nurses conduct regular, intentional rounds with patients and families to: verify care quality, address unmet needs, recognize nursing staff, identify safety concerns, and close experience gaps before they become complaints. Evidence shows consistent leader rounding improves patient satisfaction and reduces adverse events.



Healthcare Delivery

Flashcards 171–305

What is Primary Nursing as a care delivery model?

A model in which one RN has 24-hour accountability for a specific patient's care plan, coordinating all aspects of care throughout the patient's stay. It emphasizes therapeutic relationship continuity and nurse autonomy. Primary nursing is associated with higher patient satisfaction and reduced fragmentation.

What is Team Nursing?

A care model in which a team led by an RN includes LPNs and nursing assistants who are assigned a group of patients together. The RN delegates tasks based on scope of practice. It supports staffing flexibility but can fragment care if communication within the team is not structured.

What is Functional Nursing?

A task-based care model in which nurses are assigned specific functions (e.g., medication nurse, treatment nurse) across all patients on a unit rather than a defined patient group. It is efficient for high-volume low-acuity settings but reduces care continuity and holistic nursing assessment.

What is Patient-Centered Care?

A care philosophy in which patients and families are treated as partners in care decisions. Eight dimensions (IOM): respect for values, coordination of care, information and education, physical comfort, emotional support, family and friend involvement, continuity and transition, and access to care.

What is the Naylor Transitional Care Model (TCM)?

An evidence-based advanced practice nurse-led model for high-risk older adults transitioning from hospital to home.

Components include comprehensive discharge planning, home follow-up visits, telephone follow-up, and coordination with community providers. TCM reduces hospital readmissions significantly.

What is the Coleman Care Transitions Intervention?

A patient-centered model using a 'transitions coach' (APN or trained lay person) to prepare patients to be active participants in their own care transitions. The four pillars are: medication self-management, use of a personal health record, timely primary care follow-up, and knowledge of red flags.

What is EMTALA?

The Emergency Medical Treatment and Labor Act requires Medicare-participating hospitals with emergency departments to provide a medical screening exam to any patient presenting for care, regardless of insurance or ability to pay, and to stabilize any identified emergency medical condition before discharge or transfer.

What is the Triple Aim in healthcare delivery?

The Institute for Healthcare Improvement framework targeting three simultaneous goals: better health for populations, better care for individuals, and lower per capita healthcare costs. The Quadruple Aim adds a fourth goal: improving the health and well-being of healthcare providers.

What is a Care Coordination model?

A structured approach to deliberately organizing patient care activities and sharing information among all participants involved in a patient's care to achieve safer and more effective care. Evidence supports nurse-led care coordination for reducing readmissions, ED utilization, and gaps in chronic disease management.

What is a Patient-Centered Medical Home (PCMH)?

A primary care model organized around the needs of the patient, providing comprehensive, coordinated, accessible care with quality and safety focus. Key components include a personal physician, whole-person orientation, care coordination, enhanced access, quality and safety commitment, and payment reform.

What is Population Health Management?

A strategy for improving health outcomes across a defined group of individuals by identifying and managing risk, closing care gaps, and addressing social determinants of health. It requires data analytics, care management resources, community partnerships, and payment models that align incentives with outcomes.

What is Social Determinants of Health (SDOH)?

Conditions in the environments where people are born, live, learn, work, play, worship, and age that affect health outcomes. Five domains: economic stability, education access, health care access, neighborhood and built environment, and social and community context. Addressing SDOH is essential for health equity.

What is Health Equity?

The attainment of the highest level of health for all people, requiring focused societal efforts to address avoidable inequalities, historical injustices, and elimination of health disparities. In nursing leadership, it requires analyzing data by race/ethnicity, designing culturally responsive programs, and addressing structural barriers to care.

What is a readmission and why does it matter?

A readmission is a hospital admission that occurs within a defined period (typically 30 days) after a prior discharge for the same or related condition. CMS penalizes hospitals with excess readmissions under the Hospital Readmissions Reduction Program. Readmissions often reflect discharge planning gaps, inadequate post-acute support, or health literacy barriers.

What is a Discharge Planning Process?

A coordinated process beginning at admission to assess patient needs after discharge and arrange appropriate post-discharge services. Key components include multidisciplinary assessment, patient and family education, care transition arrangements (SNF, home health, outpatient follow-up), medication reconciliation, and follow-up appointment scheduling.

What is Advance Care Planning?

A process of reflection and communication about values, goals, and preferences for future medical care, particularly at the end of life. It results in advance directives (living will, healthcare proxy/durable power of attorney for health care). Nurse executives must ensure systematic advance care planning processes at their organizations.

What is Palliative Care?

Specialized medical care focused on providing relief from pain, symptoms, and the stress of serious illness at any stage of disease. It is provided alongside curative treatment, not instead of it. Evidence shows palliative care simultaneously improves quality of life and reduces healthcare costs for patients with serious illness.

What is Hospice Care?

A Medicare-covered benefit for patients with a prognosis of 6 months or less if the disease follows its expected course. Patients elect to forgo curative treatment in exchange for comfort-focused care. Provided wherever the patient calls home. Hospice includes medical, nursing, social work, spiritual, and bereavement services.

What is a Hospital-Acquired Condition (HAC)?

A condition that was not present on admission and developed during the hospital stay. HACs subject to CMS non-payment and penalties include CLABSI, CAUTI, surgical site infections, pressure injuries Stage 3/4, falls with injury, and others. HAC rates are publicly reported on Hospital Compare.

What is a Nurse-to-Patient Ratio?

The number of patients assigned to one nurse at a given time. California AB 394 mandates minimum staffing ratios by unit type (e.g., ICU 1:2, step-down 1:3, med-surg 1:5). Evidence consistently shows that lower ratios (fewer patients per nurse) are associated with lower nurse-sensitive adverse event rates.

What is Telehealth in nursing?

The delivery of health-related services using electronic information and telecommunication technologies. Modalities include synchronous video visits, asynchronous store-and-forward, remote patient monitoring, and mobile health applications. Telehealth expands access, particularly in rural and underserved communities.

What is a Community Health Worker (CHW)?

A frontline public health worker from the community they serve who provides health education, social support, and connection to resources. CHWs are effective for reducing hospital readmissions, improving chronic disease management, and addressing SDOH in high-risk populations. Evidence shows significant ROI for CHW programs.

What is Ambulatory Care Nursing?

Nursing practice in outpatient settings including clinics, physician offices, urgent care centers, and community health centers. It requires high autonomy, patient education expertise, triage skills, and care coordination. The scope includes telephone triage, chronic disease management, and patient-centered education.

What is a Care Pathway?

A structured, multidisciplinary plan that describes the essential steps in the care of a patient with a specific clinical condition. Care pathways reduce variation, support evidence-based practice, facilitate communication, and provide a framework for measuring quality. They are developed from best evidence and local expertise.

What is Motivational Interviewing (MI)?

A collaborative conversation style for strengthening a person's own motivation and commitment to change. Key techniques: open-ended questions, affirmations, reflective listening, summarizing (OARS), and exploring ambivalence. Evidence supports MI for improving adherence to chronic disease management regimens.

What is Health Literacy?

The degree to which individuals can obtain, process, understand, and use basic health information and services to make appropriate health decisions. Low health literacy is associated with worse chronic disease management, more hospitalizations, and lower adherence to treatment. Plain language and teach-back are primary interventions.

What is Teach-Back?

A communication technique in which the nurse asks the patient to explain in their own words what they have been taught, to verify understanding. Teach-back is not a test of the patient — it is a test of the nurse's teaching. It is the most evidence-supported strategy for reducing patient education failures at discharge.

What is Trauma-Informed Care?

A care approach that recognizes the widespread impact of trauma and integrates trauma awareness into clinical practice. The four Rs: Realize the prevalence of trauma, Recognize how trauma affects patients, Respond by integrating trauma knowledge, and Resist retraumatization. Relevant for all patient populations, especially behavioral health.

What is Telemetry Monitoring Nursing?

The remote continuous monitoring of cardiac rhythms through electronic transmission from the bedside to a central monitoring location.

Nurse executive responsibilities include appropriate telemetry utilization criteria, alarm management programs, telemetry nurse competencies, and overseeing alarm fatigue reduction initiatives.

What is a Medical-Surgical Nursing Unit?

An acute inpatient unit providing nursing care to adult patients with medical illnesses and pre- and post-operative surgical patients. It

represents the largest volume of inpatient nursing care. Nurse executives must manage nurse-to-patient ratios, cross-training competencies, and nursing-sensitive outcome tracking for these units.

What is an Intensive Care Unit (ICU) nurse staffing standard?

The Society of Critical Care Medicine (SCCM) and California AB 394 standards require a 1:2 nurse-to-patient ratio in adult ICUs. Evidence shows ICU nurse staffing reductions below 1:2 are associated with increased mortality, ventilator-associated pneumonia, and catheter-related infections.

What is a Rapid Response Team (RRT)?

A team of critical care specialists (typically ICU RN, respiratory therapist, and physician or NP) activated when a patient shows early signs of clinical deterioration outside the ICU. Evidence shows RRTs reduce cardiac arrest rates and hospital-acquired ICU admissions. Any staff member or family member should be able to activate an RRT.

What is an Early Warning Score (EWS)?

A physiological scoring system that assigns points based on vital sign deviations to predict patient deterioration. Examples include the Modified Early Warning Score (MEWS) and National Early Warning Score (NEWS). EWS alerts prompt nursing assessment and escalation before deterioration becomes a cardiac arrest.

What is SBAR communication?

Situation-Background-Assessment-Recommendation. A structured communication framework for conveying clinical information concisely, particularly during handoffs, escalations, and team communications. SBAR reduces ambiguity, ensures critical information is transmitted, and is a core component of TeamSTEPPS.

What is I-PASS Handoff?

A structured handoff protocol: Illness severity (stable, watcher, unstable), Patient summary, Action list, Situation awareness and contingency planning, Synthesis by receiver. Evidence shows I-PASS reduces preventable adverse events by 30% in pediatric settings. The receiver should read back their understanding.

What is a Handoff Communication?

The transfer of patient information, care responsibilities, and accountability from one caregiver to another during shift changes, inter-unit transfers, or care transitions. Effective handoffs use structured tools (SBAR, I-PASS), occur at the bedside with patient participation when possible, and include read-back verification.

What is the Donabedian Model of healthcare quality?

A framework that evaluates quality through three dimensions: Structure (resources, staffing, facilities), Process (how care is delivered, evidence adherence, workflows), and Outcome (what happened to patients — mortality, complications, satisfaction). All three dimensions must be measured to fully evaluate quality.

What is an Accountable Care Organization (ACO)?

A network of doctors and hospitals that share responsibility for providing coordinated care to Medicare patients. ACOs earn shared savings when they reduce costs below benchmarks while meeting quality standards. They shift incentives from volume to value by rewarding population health outcomes.

What is Value-Based Purchasing (VBP)?

A CMS program that adjusts hospital payments based on performance on clinical quality measures and patient experience (HCAHPS). Hospitals can earn bonuses or penalties of up to 2% of base payments. VBP measures clinical outcomes, safety, efficiency, and patient/caregiver experience.

What is HCAHPS?

The Hospital Consumer Assessment of Healthcare Providers and Systems survey is a standardized patient experience survey required by CMS. It measures 10 domains including nurse communication, physician communication, responsiveness of staff, pain management, medication communication, cleanliness, and overall rating. Results are publicly reported.

What is Patient Experience vs. Patient Satisfaction?

Patient experience measures whether specific care processes occurred (e.g., 'Did nurses explain medications before giving them?'). Patient satisfaction measures whether care met expectations (e.g., 'Were you satisfied with your care?'). Experience is more reliable for quality improvement because it measures objective processes rather than subjective expectations.

What is a Nurse-Led Clinic?

An outpatient care service managed and staffed primarily by advanced practice nurses. Nurse-led clinics provide primary care, chronic disease management, and preventive services. Evidence shows they provide equivalent quality to physician-led clinics for many conditions, with higher patient satisfaction and better follow-up.

What is the Transitional Care Model vs. Chronic Care Model?

The TCM focuses on high-risk older adults during acute-to-community transitions, led by an APN for 1-3 months. The Chronic Care Model is a broader population health framework for ongoing management of chronic conditions, involving six elements: community resources, health system organization, self-management support, delivery system design, decision support, and clinical information systems.

What is a 30-day Readmission Rate?

The percentage of patients who are readmitted to any hospital within 30 days of discharge for the same or related diagnosis. CMS publicly reports 30-day risk-standardized readmission rates for heart failure, AMI, pneumonia, COPD, hip/knee replacement, and CABG. High readmission rates trigger CMS financial penalties.

What is Interdisciplinary Rounds (IDR)?

Structured meetings at the patient's bedside or at a team station where nurses, physicians, pharmacists, social workers, and other disciplines review each patient's clinical status, care plan, goals, and discharge plan. Evidence shows IDR reduces LOS, improves discharge planning, and increases patient and family engagement.

What is the Sepsis Bundle (SEP-1)?

CMS Sepsis Care Bundle requiring: blood cultures before antibiotics, broad-spectrum antibiotics within 3 hours (1 hour for septic shock), 30 mL/kg crystalloid for hypotension/lactate ≥ 4 , reassessment for fluid responsiveness, vasopressors for MAP < 65 , cortisol for septic shock, and repeat lactate if initial ≥ 2 . SEP-1 is a CMS quality measure.

What is a Patient Safety Organization (PSO)?

A federally-certified entity that collects and analyzes patient safety data from healthcare organizations under the protections of the Patient Safety and Quality Improvement Act (PSQIA). PSQIA privilege protects PSO submissions from civil discovery to encourage open safety reporting.

What is a Culturally Competent Care?

The ability to provide care that meets the social, cultural, and linguistic needs of patients. Components include cultural awareness, knowledge, skill, encounters, and desire (Campinha-Bacote model). Nurse executives must implement CLAS Standards (Culturally and Linguistically Appropriate Services) across their organizations.

What is a Language Access Service?

Services that enable meaningful communication between healthcare providers and patients with limited English proficiency. Title VI of the Civil Rights Act requires healthcare organizations receiving federal funding to provide qualified medical interpreters. Using family members or minors as interpreters is prohibited for medical interpretation.

What is the National CLAS Standards?

The Culturally and Linguistically Appropriate Services Standards developed by DHHS.

They include 15 standards for providing culturally respectful care, language access services, communication and education, engagement, and continuous improvement.

Principal Standard: provide effective, equitable, understandable, and respectful care and services.

What is Observation Status vs. Inpatient Admission?

Inpatient admission status: physician determines patient requires inpatient care; Medicare Part A covers. Observation status: outpatient care for short stays; Medicare Part B applies; patients incur higher out-of-pocket costs; NOT eligible for SNF coverage after discharge. Status assignment has significant financial impact on patients.

What is a Jehovah's Witness and blood transfusion policy?

Competent adult Jehovah's Witnesses have the right to refuse blood transfusions based on religious beliefs. Hospitals must have a policy supporting bloodless surgery alternatives. For minor children, courts can order transfusions despite parental objection when there is an imminent threat to life.

What is the GRACE Model of Transitional Care?

Geriatric Resources for Assessment and Care of Elders — a team-based model using an NP-social worker team to provide home-based care management for low-income seniors. Evidence shows reduced hospitalizations, reduced costs, and improved quality of life. Targets patients who are frequent ED utilizers.

What is a Behavioral Health Integration Model?

A care model that co-locates or electronically integrates mental health and substance use disorder treatment with primary care. The SAMHSA-HRSA Center for Integrated Health Solutions identifies six levels of integration, from minimal collaboration to full integration. Integrated care improves access and reduces stigma.

What is a Nurse-Managed Health Center (NMHC)?

A community-based outpatient clinic led by advanced practice nurses that provides comprehensive primary care, wellness services, and health education, often in underserved communities. NMHCs are federally recognized providers under the ACA. They serve as clinical training sites for nursing students.

What is a Hospital-Based Palliative Care Consultation Team?

An interdisciplinary team (physician, APN, social worker, chaplain, nurse) that provides specialist-level symptom management, goals-of-care communication, and support for patients and families with serious illness. Evidence shows palliative care consultation reduces ICU utilization, improves symptom control, and reduces healthcare costs.

What is Medication Reconciliation?

The formal process of comparing a patient's medication orders against all medications the patient has been taking, at every care transition (admission, transfer, discharge) to identify and resolve discrepancies. Best practice combines nurse-led medication history with pharmacist clinical review. Key HAC prevention strategy.

What are nursing-sensitive indicators?

Patient outcome metrics that are primarily influenced by nursing care — including falls, fall injuries, CLABSI, CAUTI, HAPU, ventilator-associated events, and failure to rescue. NDNQI (National Database of Nursing Quality Indicators) is the primary national benchmark database for nursing-sensitive indicators.

What is a Case Manager's role in nursing?

A registered nurse who coordinates care across the continuum — assessing patient needs, planning transitions, connecting patients to resources, managing insurance authorizations, and evaluating outcomes.

Case managers prevent unnecessary hospitalization, reduce length of stay, and support complex care coordination.

What is the Patient Protection and Affordable Care Act (ACA) and nursing?

The ACA expanded insurance coverage, created Medicaid expansion, established health insurance exchanges, implemented preventive care mandates, and created value-based payment models. For nursing, the ACA expanded funding for APRN workforce development, community health centers, and nurse-managed health clinics.

What is a Designated Trauma Center?

A hospital that has been verified by the American College of Surgeons (ACS) and/or designated by the state to provide optimal care to trauma patients. Levels I-IV reflect capabilities: Level I is a comprehensive trauma center with 24/7 coverage; Level IV provides basic trauma stabilization in remote areas.

What is the Patient Right to Refuse Treatment?

A fundamental patient right grounded in the ethical principle of autonomy. Competent adult patients may refuse any medical treatment, including life-sustaining treatment, at any time. Healthcare providers must respect informed refusal, document it, ensure the patient understands consequences, and avoid abandonment.

What is Informed Consent?

A legal and ethical process in which a patient is provided with adequate information about a proposed treatment, its risks, benefits, and alternatives, and then voluntarily consents. Informed consent requires decision-making capacity, disclosure, understanding, voluntariness, and authorization. Nurses have a role in verifying that consent is truly informed.

What is a Surrogate Decision Maker?

An individual legally authorized to make healthcare decisions for a patient who lacks decision-making capacity. The standard for surrogate decision-making is substituted judgment (what the patient would have chosen). Legal surrogate hierarchy (state-specific) typically: healthcare proxy → spouse → adult child → parent → sibling → etc.

What is a Do-Not-Resuscitate (DNR) Order?

A physician order, based on patient or surrogate preference and clinical context, that instructs the care team not to perform CPR if the patient's heart stops or breathing ceases. DNR orders do not mean 'do not treat' — all other appropriate care continues. POLST is a broader portable medical order form.

What is a POLST Form?

Physician Orders for Life-Sustaining Treatment — a portable medical order signed by a physician that documents patient preferences regarding resuscitation, intubation, and artificial nutrition. Unlike advance directives (legal documents), POLST is an actionable medical order that follows the patient across care settings.

What is a Charge Nurse's role?

The charge nurse provides unit-level leadership for a specific shift: patient assignments, staffing adjustments, first-line conflict resolution, safety event response, and communication with the house supervisor. The charge nurse bridges bedside nursing and management. Charge nurses require leadership development, not just clinical seniority.

What is the nursing process (ADPIE)?

Assessment, Diagnosis, Planning, Implementation, and Evaluation. The nursing process is a cyclical, systematic framework for individualized patient care. It is the foundation of nursing practice and the basis for nursing documentation requirements in regulatory standards.

What is a Nursing Care Plan?

A written plan of nursing care developed from the nursing assessment and diagnosis that identifies patient goals, desired outcomes, nursing interventions, and evaluation criteria. Care plans must be individualized, evidence-based, and updated as patient status changes. They are required by TJC accreditation standards.

What is a Nursing Diagnosis?

A clinical judgment about an individual, family, or community response to an actual or potential health problem or life process. NANDA-I approved nursing diagnoses follow a specified format and differ from medical diagnoses, which identify disease pathology. Nursing diagnoses guide nursing interventions independent of medical orders.

What is a Never Event?

A serious, preventable patient safety event that should never occur in a healthcare setting. Defined by the National Quality Forum, examples include wrong-site surgery, surgery on the wrong patient, retained foreign body, and severe pressure injury (Stage 3/4) acquired during care. CMS does not reimburse for care related to never events.

What is a Health Impact Assessment (HIA)?

A process that estimates the potential health effects of a proposed policy, plan, or project before it is built or implemented. In nursing, HIAs inform population health advocacy by providing evidence for or against specific community development decisions that will affect the social determinants of health.

What is the Institute of Medicine (IOM) Future of Nursing report?

The 2010 IOM report 'The Future of Nursing: Leading Change, Advancing Health' made four key recommendations: nurses should practice to the full extent of their education and training, nurses should achieve higher levels of education, nurses should be full partners in healthcare redesign, and workforce planning must improve data collection. Updated in 2021.

What is a Stroke Protocol?

A time-sensitive care pathway activating a coordinated team response for acute ischemic stroke. The door-to-CT target is 25 minutes; door-to-needle (tPA administration) target is 60 minutes or less; door-to-thrombectomy is within 90 minutes. Nurse executives must ensure 24/7 stroke response capability and protocol compliance data.

What is STEMI Protocol?

ST-Elevation Myocardial Infarction protocol — a time-sensitive pathway for the fastest possible reperfusion therapy (percutaneous coronary intervention). The door-to-balloon time target is 90 minutes or less. Nurse executives must monitor this metric, ensure protocol compliance, and support team training for rapid response.

What is a Medication Administration Record (MAR)?

A comprehensive legal record of all medications administered to a patient, including date, time, dose, route, and administering nurse's signature. Electronic MARs integrated with BCMA scanning provide real-time verification and error prevention. MAR discrepancies are flagged during medication reconciliation at transitions of care.

What is a Nursing Assessment?

A systematic data collection process that forms the foundation of the nursing process. Includes history, physical examination, functional status, psychological status, social support, and patient/family priorities. Initial nursing assessments must be completed within defined timeframes per TJC standards.

What is the role of evidence in nursing practice guidelines?

Clinical practice guidelines should be based on systematic review of evidence, classified by level. Level I (systematic review/meta-analysis) is highest; expert opinion is lowest. Nurse executives must ensure that unit-level protocols are based on current evidence, reviewed regularly, and updated when evidence changes.

What is an Ethical Consult in clinical practice?

A formal consultation with the hospital ethics committee or ethics consultant to assist clinical teams and patients/families in resolving ethical dilemmas. Any member of the care team, patient, or family member should be able to request an ethics consultation. Nurse executives must ensure nurses have direct, unmediated access to ethics consultation.

What is a Visiting Nurse Association (VNA)?

A community-based nonprofit organization that provides home health nursing, physical therapy, occupational therapy, speech therapy, social work, and homemaker services to patients in their homes. VNAs are important post-acute partners for hospitals managing readmission risk and transitional care for complex patients.

What is the difference between home health and homemaker services?

Home health services are skilled nursing and therapy services covered by Medicare for homebound patients with a physician order and documented need for skilled care.

Homemaker services (personal care, housekeeping, meal preparation) are not covered by Medicare and are generally paid out-of-pocket or through Medicaid waiver programs.

What is a Hospital Incident Command System (HICS)?

A standardized management structure for hospital emergency response modeled on the Incident Command System (ICS). HICS roles include Incident Commander, Operations Chief (with medical and nursing branches), Logistics Chief, Finance/Administration Chief, and Planning Chief. All nursing leaders must know their HICS roles and the hospital's EOP (Emergency Operations Plan).

What is a Pediatric Emergency Department?

A specialized emergency department designed and staffed to meet the unique physiological, developmental, and family-centered needs of pediatric patients. Staff require pediatric-specific competencies including weight-based medication calculations, pediatric resuscitation (PALS), developmental communication strategies, and family-centered care principles.

What is a Nurse Midwife scope of practice?

Certified Nurse-Midwives (CNMs) provide primary, gynecological, and obstetric care including prenatal care, labor and delivery management, postpartum care, newborn care, family planning, and gynecological care to women across the lifespan. CNMs are licensed as APRNs and may practice independently in most states. Evidence shows CNM-led care has excellent perinatal outcomes.

What is a Maternal-Fetal Medicine (MFM) consultation?

A referral to a subspecialist (perinatologist) for high-risk obstetric conditions including multiple gestation, fetal anomalies, preterm labor, and maternal chronic disease. Nursing responsibilities include facilitating timely MFM consultation, implementing the MFM-recommended surveillance plan, and coordinating care between the obstetric and medical teams.

What is an Acute Care for Elders (ACE) unit?

A specialized inpatient unit designed to prevent hospital-acquired disability in older adults. Core features: patient-centered care, frequent mobility and function assessment, medication review for Beers criteria drugs, prepared discharge planning from admission, and an adapted physical environment (handrails, non-slip floors, bright lighting). Evidence shows ACE reduces functional decline and LOS.

What is a NICU (Neonatal Intensive Care Unit) level designation?

Level I: well-newborn nursery. Level II: specialty care for moderately ill neonates ≥ 32 weeks. Level III: subspecialty care for infants < 32 weeks or complex illness with on-site pediatric surgery access. Level IV: regional referral NICU with the highest level of subspecialty surgical and medical care. Each level has defined staffing and equipment standards.

What is an Electronic Health Record (EHR) governance?

The structures and processes for managing EHR design, configuration, access, and optimization decisions. Nursing must be represented in EHR governance to ensure that clinical documentation workflows support rather than impede nursing practice. Nurse informaticists serve as the bridge between nursing clinical needs and EHR technical configuration.

What is Clinical Decision Support (CDS)?

EHR-integrated tools that provide real-time, evidence-based alerts, reminders, and recommendations to clinicians at the point of care. Examples: sepsis screening alerts, drug-drug interaction warnings, CAUTI catheter necessity reminders. CDS reduces errors but must be carefully calibrated — excessive alerts cause alert fatigue and ignoring, defeating their purpose.

What is a Healthcare Quality Alliance?

A collaborative organization bringing together health plans, providers, and consumers to align quality measurement and public reporting. Alliances standardize quality metrics across payers to reduce provider burden and improve comparability of data. The NQF (National Quality Forum) is a prominent quality alliance that endorses standardized performance measures.

What is a Nursing Informatics Specialist?

An RN with expertise in information systems and data management who bridges clinical nursing practice and health information technology. Nursing informatics specialists design and optimize EHR workflows, train staff, analyze clinical data, support quality reporting, and participate in EHR governance. The specialty is recognized by ANCC certification (RN-BC, Informatics Nursing).

What is a Bariatric Nursing Care?

Specialized nursing care for patients with obesity, requiring: knowledge of weight-related comorbidities, bariatric-rated equipment (beds, lifts, OR tables), safe patient handling protocols, medication dose adjustments, skin care for skin folds, psychological support, and staff training to provide care with dignity. Nurse executives must ensure equipment and training are in place.

What is a specialty-specific nursing orientation?

A structured educational program for nurses new to a clinical specialty (e.g., oncology, critical care, labor and delivery) that supplements general orientation. It provides specialty-specific competency validation, medication knowledge, equipment training, and supervised clinical practice. Length and structure are defined by complexity of the specialty and new nurse experience level.

What is Hospital at Home (HaH)?

A care model delivering acute-level nursing, medical, and ancillary services in the patient's home as a substitute for inpatient hospitalization. CMS created a waiver program for HaH during COVID-19, later extended. Evidence shows HaH achieves equivalent or better clinical outcomes for eligible patients with lower cost and higher satisfaction.

What is an Interdisciplinary Care Conference?

A formal meeting of the healthcare team (RN, physician, PT, OT, SW, chaplain, dietitian, others as needed) with the patient and family to review the care plan, align on goals of care, make shared decisions about treatment direction, and coordinate discharge planning. Required for complex patients and patients in long-term care settings.

What is a nurse's role in pain management?

Nurses assess pain using validated tools (NRS, Wong-Baker, CPOT for non-verbal patients), administer prescribed analgesics using the five rights, monitor for side effects, use non-pharmacological interventions (repositioning, ice, distraction), reassess within defined intervals, document response, and escalate undertreated pain using SBAR.

What is Patient-Activated Care (PAC)?

A model in which patients and families can directly activate clinical responses — including calling the RRT via a patient-family activation arm. PAC programs are associated with reduced failure-to-rescue events and improved patient/family trust. Examples: Condition H (Help) at UPMC, allowing any patient or family member to call a rapid response.

What is a Frailty Index in nursing assessment?

A tool quantifying cumulative health deficits to identify frail older adults at high risk for adverse outcomes. The Clinical Frailty Scale (CFS, 1-9) and Edmonton Frail Scale are commonly used. Frailty assessment at admission guides care planning, prehabilitation, anesthesia risk, palliative care integration, and discharge disposition decisions.

What is a nurse's role in sepsis identification?

Early recognition using SIRS criteria, qSOFA score, or validated screening tools (Sepsis Sniffer, BestPractice Alert); timely notification to the physician; initiation of the sepsis protocol; blood culture collection before antibiotics; IV access and fluid resuscitation; medication administration per protocol; hourly reassessment and documentation of response.

What is a patient education assessment?

A structured pre-teaching assessment identifying the patient's: readiness to learn (emotional state, pain level, fatigue), health literacy, preferred learning style, language and cultural factors, existing knowledge, and barriers to learning. Effective teaching is adapted to assessment findings. Teaching without assessment produces content delivery, not learning.

What is a Critical Access Hospital (CAH)?

A rural hospital with 25 or fewer acute care beds, located more than 35 miles from the nearest hospital (or 15 miles in mountainous areas), providing emergency services around the clock. CAHs receive cost-based Medicare reimbursement (100% of allowable costs) in exchange for meeting rural access requirements.

What is a Level I Trauma Center requirement?

24/7 in-house attending trauma surgeon, 24/7 in-house anesthesia, immediate availability of operating room, board-certified trauma surgeons with high annual trauma volume, dedicated trauma ICU, verified by the American College of Surgeons. Level I trauma centers also conduct research and provide education to other trauma facilities in their region.

What is a Do Not Hospitalize (DNH) order?

An advance directive or medical order specifying that a resident of a long-term care facility should not be transferred to a hospital even in a medical emergency. DNH orders reflect a patient preference for comfort care only. Nursing home nurses must ensure DNH orders are accessible, current, and communicated at every hand-off.

What is a Nurse's Role in preventing patient falls?

Risk assessment using Morse Fall Scale at admission and with each status change, implementation of fall prevention interventions tailored to risk level (bed alarm, non-slip footwear, environmental safety, call light access, mobility assistance, toileting rounds), patient and family education, and post-fall evaluation with care plan revision.

What is a Certified Nurse Practitioner (NP) scope of practice?

NPs are APRNs who provide primary and acute care services including assessment, diagnosis, differential diagnosis, ordering and interpreting diagnostic studies, prescribing medications, and managing complex chronic and acute conditions. NP scope of practice varies by state from full practice authority to restricted collaboration requirements.

What is a Hospital's Conditions of Participation (CoPs)?

Federal standards established by CMS that hospitals must meet to participate in Medicare and Medicaid programs. CoPs cover: nursing services (including staffing, patient rights, infection control), medical staff, emergency services, patient rights, discharge planning, and quality assessment and performance improvement (QAPI). Non-compliance can result in termination from Medicare/Medicaid.

What is a Nursing Liaison role?

A nurse who serves as the communication bridge between the clinical nursing team and another department or external organization (e.g., OR liaison, transplant coordinator, case management liaison). Nursing liaisons reduce communication failures at department boundaries, coordinate complex care transitions, and ensure continuity of the nursing care plan across handoffs.

What is Evidence-Based Practice vs. Research?

Research generates new knowledge through systematic inquiry with defined methodology (RCT, cohort study, qualitative study). EBP applies existing research evidence to answer specific clinical practice questions and improve patient care. All nurses participate in EBP; nurse scientists generate new research. Both are part of a continuous knowledge development cycle.

What is a Nurse's Role in organ donation?

Identifying potential donors (patients with brain death or donation after cardiac death criteria), initiating timely referral to the Organ Procurement Organization (OPO), maintaining donor physiology stability, providing compassionate family support during donation conversations, and documenting all steps. Federal law requires hospitals to notify the OPO of all deaths and imminent deaths.

What is a Psychiatric Emergency Department?

An emergency facility specialized for the assessment and stabilization of individuals experiencing acute psychiatric crises including suicidality, psychosis, agitation, and substance withdrawal. Nurse executives must ensure: 1:1 monitoring for high-risk patients, safe environmental design (ligature-free), psychiatric emergency protocols, and staff training in de-escalation.

What is a pediatric weight-based medication calculation?

Pediatric drug doses are calculated as mg/kg of body weight rather than standard adult doses. Errors occur when: using estimated rather than verified weight, using the wrong units (pounds vs. kg), misplacing decimal points, or failing to account for maximum adult dose ceilings. All pediatric high-alert medication calculations require independent double-check.

What is Therapeutic Hypothermia (Targeted Temperature Management)?

A post-cardiac arrest intervention cooling patients to 32-36°C for 24 hours to minimize neurological injury. Nursing responsibilities include: temperature monitoring, shivering prevention, hemodynamic stabilization, skin integrity protection, and hourly neurological assessments. TTM requires critical care nursing competency and is most effective when initiated early.

What is a Patient Advocate/Patient Representative role?

A hospital employee who assists patients in exercising their rights, resolving complaints, navigating the healthcare system, and accessing services. The patient advocate functions independently from the clinical team and reports to hospital administration. Nurse executives must ensure that nursing staff respect the patient advocate's role without defensiveness.

What is the Magnet Model's Exemplary Professional Practice component?

One of five Magnet model components, requiring evidence that nurses practice to the full extent of their education and authority, that professional practice models are in place and used, that interprofessional collaboration is demonstrated, that nurses engage in ethics consultations, and that patient outcome data demonstrates high-quality nursing care.

What is a nurse-midwife vs. labor and delivery nurse scope difference?

A CNM independently manages labor, delivers babies, manages obstetric complications, and prescribes within their scope. An L&D; RN implements the CNM or physician's plan of care, monitors labor progress, administers medications, provides support, documents, and escalates complications. The L&D; RN cannot independently manage labor or deliver without CNM or physician order and direction.

What is a multidisciplinary discharge planning team?

A team including the bedside RN, case manager, social worker, physician, pharmacist, therapists, and patient/family that collaborates to ensure the patient is prepared for safe discharge. Effective discharge planning begins at admission, identifies barriers early, arranges post-acute services before discharge day, and verifies patient/family understanding before release.

What is opioid stewardship in nursing?

A program to optimize opioid prescribing and administration to minimize harm (addiction, diversion, overdose) while effectively managing pain. Nursing components include: multimodal pain assessment, non-opioid alternatives, PRN opioid documentation with reassessment, naloxone co-prescribing education, safe disposal instructions, and patient/family education on opioid risks.

What is a patient's right to pain management?

Patients have the right to have their pain assessed and treated appropriately. The Joint Commission and CMS recognize pain management as a patient right. However, this does not mandate a specific pain score target or guarantee opioid prescriptions. Nurses must document pain assessment, interventions, and response while balancing pain management with opioid safety.

What is a nurse staffing crisis plan?

A documented contingency plan for extreme staffing shortages (e.g., mass illness, natural disaster, walkout). Components include: tiered response levels, internal redeployment criteria, agency/contract nurse activation process, scope expansion authorization, diversion criteria, and communication cascade to leadership, medical staff, and regulatory agencies.

What is a vulnerable population in healthcare delivery?

Individuals with characteristics that limit their ability to protect their own health interests. Includes: children, elderly, pregnant women, people with disabilities, limited English proficiency, low income, LGBTQ+ individuals, people experiencing homelessness, and incarcerated individuals. Nursing care for vulnerable populations requires tailored communication, advocacy, and culturally competent approaches.

What is a Nurse's role in domestic violence screening?

Universal screening for intimate partner violence (IPV) is recommended for all adult patients in primary care and obstetric settings. Nurses screen using validated tools (HITS, WAST), provide non-judgmental support, document findings per policy, provide safety resources, and report per mandatory reporting laws when required (e.g., injuries from weapons, suspected child abuse).

What is a Public Reporting of Hospital Quality Data?

Federal transparency initiatives requiring hospitals to publicly report quality metrics. CMS Hospital Compare publishes: HCAHPS scores, HAI rates, readmission rates, mortality rates, and value-based purchasing scores. Public reporting enables patient choice and creates competitive accountability for quality improvement.

What is a Nurse Practitioner-led Urgent Care Center?

An outpatient setting where NPs independently manage low-to-moderate acuity conditions without routine physician oversight. Evidence shows NP-led urgent care provides equivalent quality to physician-led care for these conditions, with higher patient satisfaction and comparable safety outcomes. NP scope requirements vary by state.

What is a nurse's role in medication-assisted treatment (MAT)?

Supporting patients receiving buprenorphine, methadone, or naltrexone for opioid use disorder: administering medications per order, monitoring for diversion or compliance issues, providing psychosocial support, educating on overdose prevention (naloxone), and coordinating with addiction medicine and behavioral health services. Non-judgmental care is essential.

What is Patient Education Documentation?

A required documentation element capturing: what was taught, the method used, patient response (verbalized understanding, demonstrated skill, expressed barriers), plan for follow-up teaching, and involvement of family/caregiver. TJC requires documentation of patient education. Inadequate documentation creates liability risk and continuity of care gaps.

What is a Wound, Ostomy, and Continence (WOC) Nurse?

A certified specialty nurse with advanced expertise in wound care, ostomy management, and continence care. WOC nurses consult on complex pressure injuries, surgical wounds, fistulas, ostomy appliance selection, and urinary/fecal incontinence management. Nurse executives should ensure WOC nurse access for complex wound patients to reduce HAPU rates and improve healing outcomes.

What is a Charge Nurse's responsibility during a code blue?

Ensure the code team is alerted immediately, coordinate access to the crash cart and defibrillator, clear the room of non-essential personnel and equipment, notify the attending physician, facilitate family notification and support, assign a nurse as scribe for real-time documentation, and prepare to communicate with the incoming transfer team or intensivist.

What is Patient Safety Indicator (PSI) measure?

AHRQ-developed indicators measuring potentially preventable complications and iatrogenic injuries occurring during hospitalization. PSIs include: iatrogenic pneumothorax, postoperative respiratory failure, accidental puncture, postoperative DVT/PE, and foreign body left after procedure. CMS uses PSI data in Value-Based Purchasing adjustments.

What is a Clinical Pharmacy Specialist?

A pharmacist with advanced clinical training who is integrated into the patient care team to review medication regimens, prevent drug-drug interactions, recommend dosing adjustments, manage complex pharmacotherapy, and provide pharmacist-led medication reconciliation. Nurse-pharmacist collaboration significantly reduces medication errors in high-acuity settings.

What is a Universal Protocol (TJC)?

A TJC requirement for preventing wrong site, wrong procedure, and wrong person surgery. Three components: pre-procedure verification process, marking the operative site, and a time out immediately before the procedure. The universal protocol applies to all invasive procedures performed in any setting, not just the operating room.

What is a patient's right to a second opinion?

Patients have the right to seek a second medical opinion before consenting to a recommended procedure or treatment. Nurses support this right by providing non-judgmental information, facilitating referrals, and ensuring that the second opinion process does not result in care delays that harm the patient. Insurance plans must cover second opinions for serious diagnoses.

What is a Skilled Nursing Facility (SNF) Medicare Benefit?

Medicare Part A covers SNF care for up to 100 days per benefit period for patients who: have a qualifying 3-day inpatient hospital stay (not observation), require skilled nursing or therapy services, and meet the medical necessity criteria. Days 1-20 are covered at 100%; days 21-100 require a daily copay. Patients must be reassessed regularly to maintain eligibility.

What is a Behavioral Health Nursing Model?

A nursing model that addresses patients' psychological, emotional, and behavioral health needs across settings. It requires: trauma-informed care, de-escalation skills, therapeutic communication, safety assessment for self-harm, psychotropic medication management, and collaborative care with behavioral health specialists. Behavioral health competencies are required in all nursing settings, not just psychiatric units.

What is Patient Handoff Literacy?

The ability of nurses to communicate patient-specific clinical information accurately, completely, and concisely during transitions of care. Handoff literacy requires knowledge of the relevant clinical facts, skill in structured communication tools (SBAR, I-PASS), situational awareness of recipient needs, and reading-back confirmation. Handoff literacy is a foundational patient safety competency.

What is a Care Transition Nurse?

An RN trained specifically in managing patients' transitions between care settings (hospital to home, hospital to SNF, ED to inpatient). Care transition nurses contact patients post-discharge, identify and resolve transition problems, reconcile medications, verify follow-up appointment completion, and coordinate with community providers to prevent readmissions.



Quality & Safety

Flashcards 306–425

What is PDSA?

Plan-Do-Study-Act is a rapid-cycle quality improvement framework. Plan: identify the problem and design a test. Do: implement on a small scale. Study: analyze results and compare to predictions. Act: adopt, adapt, or abandon based on findings. PDSA cycles are iterative and accelerate evidence-based improvement.

What is DMAIC in Six Sigma?

Define (problem statement, project charter, VOC), Measure (baseline data, process mapping), Analyze (root cause analysis, fishbone diagram, Pareto), Improve (generate solutions, pilot), Control (standardize solution, monitor). DMAIC is the structured improvement roadmap for Six Sigma projects.

What are the 5 Principles of Lean?

Identify value (from the customer's perspective), Map the value stream (identify all steps), Create flow (eliminate interruptions), Establish pull (produce only when needed), Seek perfection (continuous improvement). In healthcare, Lean identifies and eliminates waste to improve quality and flow.

What are the 8 Types of Waste in Lean (DOWNTIME)?

Defects, Overproduction, Waiting, Non-utilized talent, Transportation, Inventory, Motion, Extra-processing. DOWNTIME is the healthcare-adapted mnemonic. Nurse executives target waste elimination to reduce costs, improve efficiency, and free nursing time for direct patient care.

What is a Value Stream Map?

A Lean tool that visually maps all steps (value-added and non-value-added) in a process from start to finish. It identifies waste, delays, and bottlenecks. The current-state map shows how work is done today; the future-state map shows the improved design. Used to guide redesign of high-waste clinical workflows.

What is Just Culture?

A framework for responding to safety events that differentiates between human error (console), at-risk behavior (coach), and reckless behavior (punish). It holds individuals accountable for behavioral choices while acknowledging system-level factors. Just Culture prevents both over-punishment and under-accountability.

What is a High Reliability Organization (HRO)?

An organization that operates in high-hazard domains with complex systems and consistently avoids catastrophic outcomes.

Five HRO principles: preoccupation with failure, reluctance to simplify, sensitivity to operations, commitment to resilience, and deference to expertise. Examples: aircraft carriers, nuclear power plants, and aspirational healthcare models.

What is Preoccupation with Failure (HRO)?

An HRO mindset in which individuals treat small signals of system weakness as potential precursors to catastrophic failure. It requires actively seeking problems, valuing near-miss reports, and never assuming that past success guarantees future safety. The opposite is complacency.

What is Sensitivity to Operations (HRO)?

Maintaining real-time awareness of how systems are actually functioning on the front line. It requires leadership presence on the floor (rounding), front-line huddles, and communication channels that bring operational concerns to leadership without filtering. Disconnection from operations is a precursor to catastrophic failure.

What is Deference to Expertise (HRO)?

In a crisis or safety-critical situation, decision-making authority should shift to the person with the most relevant knowledge and expertise, regardless of hierarchical rank. A junior nurse identifying a critical safety issue should be heard. Organizational rank deference in the face of front-line expertise is an HRO failure.

What is a Swiss Cheese Model of Error?

James Reason's model showing that safety systems have layers of defense, each with holes (weaknesses). When the holes in multiple layers align, a harm event occurs. It supports systems thinking: individual errors are the last line of defense. Effective safety design requires multiple redundant layers.

What is Failure Mode and Effects Analysis (FMEA)?

A proactive risk assessment tool that identifies all potential failure modes in a process, assesses their likelihood, severity, and detectability, calculates a Risk Priority Number ($RPN = \text{Severity} \times \text{Occurrence} \times \text{Detectability}$), and prioritizes corrective actions. FMEA is conducted before an adverse event, not after.

What is RPN and how is it calculated?

Risk Priority Number = Severity × Occurrence × Detectability, each scored 1-10. Higher RPN = higher priority for intervention. However, a failure mode with maximum severity (score 10) should be prioritized for action regardless of RPN because the potential for catastrophic harm outweighs other factors.

What is a Root Cause Analysis (RCA)?

A retrospective systematic investigation of a sentinel or serious safety event to identify the contributing factors and underlying system failures. RCA uses fishbone diagrams, 5 Whys, and process maps. Output is an action plan with specific corrective actions, accountable parties, and timelines for implementation.

What is the 5 Whys technique?

A root cause analysis method that iteratively asks 'why' five or more times to drill down from a surface symptom to its underlying cause. It helps identify system-level causes rather than stopping at proximate individual causes. Example: Why did the nurse give the wrong dose? Why was the order unclear? etc.

What is CLABSI and how is the rate calculated?

Central Line-Associated Bloodstream Infection. Rate = (Number of CLABSI events ÷ Number of central line days) × 1,000. NHSN reports the Standardized Infection Ratio (SIR) = observed ÷ predicted infections. CLABSI prevention bundles include hand hygiene, maximal barrier precautions, chlorhexidine antisepsis, and daily removal assessment.

What is CAUTI and its prevention bundle?

Catheter-Associated Urinary Tract Infection.

Prevention bundle: avoid unnecessary catheter insertion (use alternatives), daily necessity review, use aseptic technique for insertion, maintain closed drainage system, position bag below bladder, and remove catheter as soon as clinically possible.

CAUTI is a CMS Hospital-Acquired Condition.

What is HAPU (Hospital-Acquired Pressure Injury)?

A pressure injury that was not present on admission and developed during the hospitalization. Prevention bundle includes: skin assessment within 8 hours of admission, Braden Scale scoring, repositioning every 2 hours or per individualized plan, moisture management, nutrition optimization, and pressure-redistributing surfaces. HAP Stage 3/4 is a CMS Never Event.

What is the Braden Scale?

A validated 6-item tool for assessing pressure injury risk: Sensory Perception, Moisture, Activity, Mobility, Nutrition, and Friction/Shear. Total score ranges from 6-23; lower scores indicate higher risk. Score ≤ 18 typically triggers a prevention protocol. It must be completed at admission and with each status change.

What is a Hand Hygiene Compliance Rate?

The percentage of hand hygiene opportunities in which the correct technique was performed. WHO 5 Moments: before patient contact, before aseptic task, after body fluid exposure, after patient contact, after contact with patient surroundings. Direct observation is the gold standard but subject to Hawthorne effect. Electronic monitoring provides more objective data.

What is the Surgical Safety Checklist (WHO)?

A three-part checklist with Sign In (before anesthesia), Time Out (before incision), and Sign Out (before patient leaves OR). It verifies: patient identity, procedure, site, consent, equipment, allergies, and specimen labeling. Evidence shows it reduces surgical mortality and complications when used with fidelity.

What is a Time Out in surgery?

A mandatory pause immediately before beginning a procedure in which the entire team verbally confirms: patient identity, procedure type, surgical site, patient position, implants/equipment, and any concerns. All team members must actively participate (not just be present). Passive presence does not count as a verified time out.

**What is the TeamSTEPPS
Two-Challenge Rule?**

A patient safety rule: when a team member raises a concern about a safety issue, they must state it at least twice assertively if not acknowledged. If the concern is still not addressed after two challenges, the team member should take a stronger action (e.g., escalate to a supervisor or invoke stop-the-line authority).

What is CUS in TeamSTEPPS?

A safety assertion tool: 'I am Concerned. I am Uncomfortable. This is a Safety issue.' CUS is used when a team member needs to stop an unsafe action. Using 'safety issue' explicitly signals that the team must pause and address the concern before proceeding. CUS overrides hierarchy deference in the moment.

What is a Safety Huddle?

A brief, structured daily meeting at the unit or system level to proactively identify safety risks, anticipate challenges, and coordinate real-time awareness across the team. Safety huddles are a core element of HRO daily management. They are distinct from clinical handoffs and focus on situational awareness and risk anticipation.

What is the AHRQ Hospital Survey on Patient Safety Culture (HSOPSC)?

A validated survey measuring staff perception of patient safety culture. Composite dimensions include: Teamwork Within Units, Supervisor/Manager Expectations and Actions Promoting Safety, Organizational Learning and Continuous Improvement, Management Support for Patient Safety, and Nonpunitive Response to Error. Results benchmark across 1,000+ hospitals.

What is Nonpunitive Response to Error?

A HSOPSC composite dimension measuring whether staff believe errors are held against them and whether mistakes are recorded but not used against the person who made them.

It is one of the lowest-scoring composites nationally. Nonpunitive response is essential for voluntary reporting and safety learning.

What is an Adverse Drug Event (ADE)?

Any harm to a patient resulting from drug therapy, including side effects, overdose, underdose, allergic reactions, and medication errors. ADEs are the most common type of inpatient adverse event. BCMA systems, pharmacist reconciliation, and clinical decision support alerts are primary prevention strategies.

What is Barcode Medication Administration (BCMA)?

A safety technology that requires nurses to scan their ID, the patient's wristband barcode, and the medication barcode before administration to verify the 5 Rights (patient, drug, dose, route, time). BCMA reduces medication administration errors by 50-85%. Workarounds defeat the system's safety function.

What is High-Alert Medication?

A drug that bears a heightened risk of causing significant patient harm when used in error. ISMP High-Alert Medication list includes: insulin, anticoagulants, opioids, chemotherapy, potassium chloride, neuromuscular blocking agents, and concentrated electrolytes. High-alert medications require independent double-check and special storage.

What is an Independent Double-Check?

A medication safety process in which two clinicians independently (without conferring first) perform the same calculation or verification before administering a high-alert medication. True independence is essential — if one clinician shows the other their work first, the check is not independent and provides false safety assurance.

What is Infection Prevention in nursing?

A systematic program to identify, prevent, and control healthcare-associated infections (HAIs). Core elements: surveillance (NHSN reporting), bundle compliance (CLABSI, CAUTI, VAP, SSI), hand hygiene, isolation precautions, environmental cleaning, antibiotic stewardship, and outbreak investigation.

What are Standard Precautions?

Infection control measures applied to all patients regardless of diagnosis: hand hygiene, gloves for contact with blood/body fluids, gowns for splash risk, masks/eye protection when appropriate, safe sharps handling, respiratory hygiene, and proper waste disposal. Standard Precautions replaced Universal Precautions in 1996.

What are Transmission-Based Precautions?

Additional precautions for patients with known or suspected contagious pathogens. Contact (e.g., MRSA, VRE, C. diff): gloves and gown. Droplet (e.g., influenza, COVID): surgical mask. Airborne (e.g., TB, measles, varicella): N95 respirator and negative pressure room. Can be combined when transmission route is unknown.

What is Antimicrobial Stewardship?

A coordinated program to optimize antimicrobial therapy to improve patient outcomes, reduce resistance, minimize C. diff, and reduce costs. Core strategies: prospective audit and feedback, formulary restriction, prior authorization, rapid diagnostics, de-escalation, and IV-to-oral conversion. Nurse executives support stewardship by integrating it into nursing practice governance.

What is a Pressure Injury Classification (NPUAP)?

Stage 1: Non-blanchable erythema of intact skin. Stage 2: Partial thickness skin loss. Stage 3: Full thickness tissue loss (subcutaneous visible). Stage 4: Full thickness tissue loss with exposed bone/tendon/muscle. Unstageable: obscured by eschar. Deep Tissue Injury: purple/maroon intact skin with blood-filled blister.

What is the Aldrete Score?

A post-anesthesia recovery scoring system: Activity (0-2), Respiration (0-2), Circulation (0-2), Consciousness (0-2), Oxygen Saturation (0-2). Total score 0-10; score ≥ 9 typically required for Phase I recovery discharge. It guides nurse assessment of readiness to transfer from the PACU to lower-acuity care.

What is a Rapid Response System (RRS)?

A hospital-wide system for identifying and responding to patients with early clinical deterioration before cardiac arrest. Components: afferent limb (detection, calling criteria), efferent limb (RRT team response), administrative limb (data collection, performance improvement), and owner limb (leadership governance). Evidence shows RRS reduces non-ICU cardiac arrest.

What is the Failure to Rescue (FTR)?

The rate at which patients die after developing a serious complication (e.g., pneumonia, DVT/PE, sepsis, shock) during hospitalization that could potentially have been treated and survived. FTR is a nursing-sensitive indicator: well-staffed units with more educated nurses have lower FTR rates. It reflects surveillance and escalation competency.

What is a Run Chart?

A graph of data over time used to detect patterns of improvement, deterioration, or unusual variation. Special cause signals on a run chart: 8 consecutive points above or below the median, a trend of 6+ consecutive increasing or decreasing points. Run charts are used in PDSA cycles to track process changes.

What is a Statistical Process Control (SPC) Chart?

A run chart with statistically calculated upper and lower control limits (3 standard deviations from the mean). Common cause variation falls within the control limits — the process is stable and predictable. Special cause variation falls outside limits — something changed. SPC charts distinguish random noise from real signals.

What is Common Cause vs. Special Cause Variation?

Common cause variation: random variation inherent to the process, showing as stable data within control limits. No intervention needed — only system redesign can reduce it. Special cause variation: non-random variation caused by an identifiable factor outside the process. Requires investigation and targeted correction.

What is a Fishbone Diagram (Ishikawa)?

A visual root cause analysis tool that organizes potential causes of a problem by category. In healthcare, the 6Ms framework is used: Man (people), Machine (equipment), Method (process), Measurement (data), Material (supplies), and Milieu (environment). Each potential cause is examined to identify root causes.

What is a Pareto Chart?

A bar graph that ranks causes of a problem from most frequent to least frequent, with a cumulative percentage line. The Pareto Principle (80/20 rule) suggests 80% of problems come from 20% of causes. It helps quality teams prioritize interventions by focusing on the most frequent contributing factors first.

What is the SQUIRE 2.0 Guidelines?

Standards for Quality Improvement Reporting Excellence — a framework for reporting quality improvement work in peer-reviewed publications. It requires transparent description of the intervention's context, theory, methods, study design, results (including unintended consequences), and lessons learned. Used for publishing QI work.

What is the Hierarchy of Evidence in EBP?

From strongest to weakest: Level I (systematic review/meta-analysis of RCTs), Level II (individual RCT), Level III (controlled trial without randomization), Level IV (cohort or case-control study), Level V (systematic review of descriptive studies), Level VI (individual descriptive study), Level VII (expert opinion/consensus).

What is an Evidence-Based Practice (EBP) Model?

A framework for integrating research evidence, clinical expertise, and patient preferences into practice decisions. Common models: Iowa Model (triggers, appraise evidence, pilot, disseminate), Johns Hopkins, ACE Star, STETLER Model. The EBP process includes asking a clinical question (PICO), finding evidence, appraising it, applying it, and evaluating outcomes.

What is a PICO Question?

A structured clinical question format: Population (who is the patient?), Intervention (what treatment/intervention?), Comparison (what is the alternative?), Outcome (what is the desired result?). PICO guides literature searching and makes the clinical question explicit and searchable. Sometimes T (time) is added: PICOT.

What is Sentinel Event Policy (Joint Commission)?

TJC defines a sentinel event as an unexpected occurrence involving death or serious physical or psychological harm. Accredited organizations are expected to conduct a comprehensive systematic analysis (root cause analysis) and create an action plan within 45 days of the event. TJC has a voluntary reporting program.

What is the Joint Commission's NPSG (National Patient Safety Goals)?

Annual requirements from TJC addressing the most common patient safety concerns. Current NPSGs include: improving patient identification accuracy (two identifiers), improving communication effectiveness, medication safety, alarm safety, HAI reduction, clinical alarm safety, and suicide risk reduction.

What is the NHSN (National Healthcare Safety Network)?

CDC's internet-based surveillance system for tracking HAIs. Hospitals report CLABSI, CAUTI, VAP, SSI, MRSA bacteremia, and C. diff infection rates. NHSN data is used to calculate Standardized Infection Ratios (SIR) for national benchmarking and CMS quality reporting requirements.

What is a Standardized Infection Ratio (SIR)?

$$\text{SIR} = \text{Observed infections} \div \text{Predicted infections (adjusted for risk factors)}$$

SIR < 1.0 = fewer infections than predicted (better than expected). SIR > 1.0 = more infections than predicted (worse than expected). SIR = 1.0 = performance matches risk-adjusted prediction. SIR is the primary NHSN benchmarking metric.

What is Alarm Fatigue?

A state in which clinicians become desensitized to clinical alarms due to high alarm volume, leading to delayed response or ignoring alarms, including life-threatening ones. Causes: inappropriate alarm thresholds, non-individualized parameters, nuisance alarms. TJC NPSG #6 requires alarm management programs in all accredited hospitals.

What is an Alarm Management Program?

A system-level initiative to reduce non-actionable clinical alarms while maintaining safety for true deterioration events. Components include: individualized alarm parameter setting, regular electrode changes, alarm limit assessment per patient, escalation protocols, alarm technology optimization, and education for nurses on alarm management principles.

What is Restraint Policy in accredited hospitals?

TJC and CMS require: least-restrictive alternative tried first; physician order with specific timeframes; continuous monitoring; documentation of behavior justification; regular reassessment; and family notification. Chemical restraints (sedation used to restrict freedom of movement) have the same documentation requirements as physical restraints.

What is the Nursing Quality Indicators (NDNQI)?

The National Database of Nursing Quality Indicators is the largest nursing-specific quality database in the U.S. It collects unit-level data on nursing-sensitive indicators including falls, HAPU, CLABSI, CAUTI, nurse satisfaction, and skill mix. Hospitals submit quarterly data and receive benchmarked reports comparing their unit to national peers.

What is an Incident Report (Safety Event Report)?

A confidential internal document used to report an unexpected event that affected or could have affected patient, visitor, or employee safety. It is used for quality improvement analysis, not for disciplinary action. Under most state laws and PSO protections, incident reports are protected from legal discovery.

What is the Joint Commission Speak Up Campaign?

A patient safety initiative encouraging patients and families to speak up if they have questions or concerns about their care, observe something that seems wrong, or need help communicating. It supports a culture of safety by treating patients as active participants in their own care team.

What is a Plan of Correction (PoC)?

A formal written response to regulatory deficiencies identified during a CMS or state health department survey. The PoC must address: immediate corrective action, systemic root cause, how the deficiency was identified, ongoing monitoring mechanism, and the responsible party for each cited deficiency. Must be submitted within defined timeframes.

What is an IRB (Institutional Review Board)?

A federally mandated committee that reviews research involving human subjects to protect their rights, safety, and welfare. Under the Common Rule, research is classified as exempt (minimal risk, standard practice), expedited (greater than minimal risk, limited harm potential), or full board review (more than minimal risk). IRB approval is required before research begins.

What is Exempt vs. Expedited IRB Review?

Exempt: Research involving normal educational practices, survey/interview procedures with minimal risk, or secondary use of existing data. Expedited: Research involving minor risks (blood draws, existing data with identifier risk, behavioral studies). Full Board: Research involving greater than minimal risk, vulnerable populations, or deception.

What is a Clinical Practice Guideline (CPG)?

A systematically developed document based on best evidence that provides recommendations to assist practitioners in clinical decision-making for specific clinical circumstances. CPGs are developed by professional organizations (e.g., ANA, AHRQ, specialty societies) and must be adapted to local context for implementation.

What is Lean 5S Methodology?

A workplace organization method: Sort (remove unnecessary items), Set in Order (organize remaining items for efficiency), Shine (clean and inspect), Standardize (develop standards), Sustain (maintain the first 4S with discipline). 5S reduces waste from motion and searching, and creates visual management for immediate problem identification.

What is an ISMP High-Alert Medication Double-Check?

ISMP recommends independent double-check for high-alert medications where one clinician miscalculates or prepares the drug and another independently verifies without seeing the first calculation. Both must physically check: drug, concentration, volume, pump setting, and patient. Double-check reduces high-alert medication error rates significantly.

What is the Medication Reconciliation Process at Transitions?

At admission: obtain complete medication history (BPMH), compare to orders, resolve discrepancies. At transfer: compare current orders to BPMH, add/discontinue as needed. At discharge: reconcile all changes, create final medication list, provide patient education, and fax/transmit to next provider. Best practice is nurse+pharmacist collaboration.

What is an Environmental Rounds in quality safety?

Systematic inspection of patient care environments to identify safety hazards, compliance issues, and equipment problems before they cause harm. Environmental rounds typically follow a standardized checklist addressing: medication security, crash cart readiness, fall hazards, fire safety compliance, and equipment maintenance. Results are trended for improvement.

What is a Mock Tracer in quality accreditation?

An internal simulation of a Joint Commission accreditation tracer — following a patient's care journey through the organization to test compliance with standards. Mock tracers identify compliance gaps before the actual survey. They are most effective when conducted with the same rigor and surprise as actual surveys.

What is Drug Diversion in nursing?

The theft or misuse of controlled substances by healthcare workers. Detection methods include: auditing controlled substance dispensing logs, monitoring waste discrepancy patterns, patient pain complaints despite documented adequate analgesia, and random drug testing. Reporting obligations vary by state but typically include the state board of nursing and DEA.

What is a Nursing Quality Dashboard?

A visual summary of key nursing quality metrics for leadership decision-making. It typically includes: nursing-sensitive outcome rates (vs. NDNQI benchmarks), process compliance rates (bundle adherence), patient experience scores (HCAHPS), workforce indicators (vacancy, turnover, overtime), and safety event rates.

What is an Infection Surveillance Definition?

A standardized criteria set used to identify and count healthcare-associated infections consistently. NHSN definitions must be used for reportable HAI metrics. Using non-standard definitions creates non-comparable data. Definition changes (e.g., CLABSI exclusion criteria) require transparent disclosure in quality reports.

What is a Safety Culture Assessment?

A systematic measurement of staff perceptions about the organization's commitment to safety. AHRQ HSOPSC is the most widely used tool. Dimensions include: event reporting frequency, communication openness, supervisor actions promoting safety, nonpunitive response to error, and teamwork within and across units. Results guide culture improvement initiatives.

What is a Workplace Violence Prevention Program?

A system-level program addressing violence against healthcare workers. Components: behavioral flag system in EHR, environmental design controls, de-escalation training, incident reporting process without retaliation, worker's compensation and psychological support after assault, and clear organizational message that violence is not acceptable.

What is a Post-Acute Care Facility (SNF, LTACH, IRF)?

Skilled Nursing Facility (SNF): provides skilled nursing and rehabilitation for patients requiring continued care after acute hospitalization. Long-Term Acute Care Hospital (LTACH): provides prolonged acute care (average LOS ≥ 25 days) for medically complex patients. Inpatient Rehabilitation Facility (IRF): intensive rehab (minimum 3 hours/day) for patients after acute events.

What is a Safety Event Classification (NCC MERP)?

National Coordinating Council for Medication Error Reporting and Prevention classifies events A-I: A (capacity to cause error), B (error occurred, no harm), C (error occurred, monitoring needed), D (error occurred, intervention to prevent harm), E-H (varying levels of harm), I (error contributed to death). Used to prioritize investigation depth.

What is an Antibiotic Timeout?

A reassessment of antibiotic therapy at 24-72 hours after initiation to determine whether de-escalation is appropriate based on culture results, clinical response, and current diagnostic information. Antibiotic timeouts are a core antimicrobial stewardship strategy that reduces broad-spectrum antibiotic overuse and C. diff risk.

What is a Patient Safety Committee?

A multidisciplinary governance structure responsible for oversight of patient safety programs, review of serious safety events, tracking of near-miss trends, monitoring implementation of corrective actions, and reporting safety performance to the board quality committee. Nurse executives typically serve as standing members or co-chairs.

What is a Care Bundle?

A small set of evidence-based practices (typically 3-5) that when performed together consistently produce better outcomes than each practice performed individually. Bundle elements must all be reliably performed to achieve the synergistic effect. Examples: CLABSI bundle (CHABLIS), VAP bundle, sepsis bundle (SEP-1).

What is the VAP (Ventilator-Associated Pneumonia) Prevention Bundle?

Core elements: head of bed elevation 30-45°, daily interruption of sedation, daily assessment for extubation readiness, peptic ulcer disease prophylaxis, deep vein thrombosis prophylaxis, and oral care with chlorhexidine. VAP significantly increases morbidity, mortality, and LOS. VAE (Ventilator-Associated Event) has replaced VAP in NHSN reporting.

What is a Nursing Sensitive Quality Measure?

A patient outcome that is primarily influenced by the quality of nursing care. NDNQI nursing-sensitive measures include: falls (with and without injury), hospital-acquired pressure injuries, CLABSI, CAUTI, RN education and certification, nurse satisfaction, and nursing hours per patient day. These measures are used to evaluate the impact of nursing practice on outcomes.

What is a Failure to Rescue Rate Benchmark?

The percentage of patients who die after developing a major complication. NDNQI provides unit-level benchmarks by hospital type. Lower nurse staffing ratios and lower nurse education levels are independently associated with higher FTR rates. Nurse executives use FTR data to justify safe staffing investment and evaluate nursing surveillance effectiveness.

What is a Quality Improvement Charter?

A formal document authorizing and defining a quality improvement project. Charter elements: problem statement, AIM statement (Specific, Measurable, Achievable, Relevant, Timebound), team membership, scope, timeline, resources, and leadership sponsor.

A signed charter signals organizational commitment and helps teams stay focused on defined scope.

What is an AIM Statement in quality improvement?

A written statement of the specific improvement goal for a QI project, including: what will improve, by how much, in what timeframe, and in what population. Example: 'Reduce CAUTI rate from 1.8/1,000 catheter days to 0.5/1,000 catheter days in ICU patients within 6 months.' SMART AIM statements provide clarity and enable measurement.

What is a Process Map?

A visual diagram of the sequence of steps in a process, including decision points and handoffs. Process maps reveal complexity, redundancy, and points of vulnerability. Current-state process maps (as done) often reveal unexpected steps not in the formal procedure. Future-state maps show the redesigned, improved process.

What is the Lean concept of Kaizen?

A Japanese term meaning 'continuous improvement' or 'change for the better.' In Lean, kaizen events (also called rapid improvement events or Rapid Process Improvement Workshops) are intense 3-5 day structured workshops bringing together a cross-functional team to analyze and redesign a specific process with immediate implementation of improvements.

What is a Surgical Site Infection (SSI) bundle?

Evidence-based practices: appropriate antibiotic prophylaxis (timing, agent, re-dosing), glucose control perioperatively, normothermia maintenance, appropriate hair removal (clipper not razor), proper surgical site prep (chlorhexidine-alcohol), and standardized sterile technique. SSI bundles reduce infection rates significantly when implemented with high fidelity.

What is a Pressure Injury Prevalence Study?

A point-in-time cross-sectional survey of all patients on a defined unit or facility to determine the proportion with pressure injuries present (prevalence). NDNQI conducts quarterly prevalence studies.

Prevalence studies include both community-acquired (present on admission) and hospital-acquired pressure injuries.

Incidence studies track only new hospital-acquired injuries.

What is Error Proofing (Poka-Yoke)?

A design approach that makes errors physically impossible or immediately detectable. Examples: Luer-lock connectors that prevent misconnection, color-coded IV tubing for specific routes, barcode scanning that alerts before wrong drug administration. Error proofing is the highest level of defect prevention in safety system design.

What is a Healthcare-Associated Infection (HAI) Bundle Compliance Rate?

The percentage of bundle elements completed for all eligible patients. True bundle compliance requires all elements to be present for each patient — not an average of individual element rates. For example, 90% compliance on each of 5 CLABSI bundle elements does not equal 90% bundle compliance; joint probability is $0.9^5 = 59\%$ full compliance.

What is a Lean Gemba Walk?

A structured leadership practice of going to where work is done (gemba = 'the real place' in Japanese) to observe processes, identify waste, ask questions, and coach staff on problem-solving. Gemba walks are distinct from rounding — the purpose is learning and coaching, not inspection. Gemba walks are a foundational practice of Lean leadership.

What is a Sentinel Event Alert (SEA)?

A Joint Commission publication that alerts healthcare organizations to specific safety issues identified through sentinel event reporting and analysis. SEAs provide root cause data and recommended preventive actions. They are issued when TJC identifies patterns in adverse event root causes that require urgent system-level attention. Organizations are expected to review SEAs and assess applicability to their practice.

What is an Escalation Protocol?

A structured communication process for nurses to report patient deterioration or safety concerns to progressively higher levels of clinical authority when responses at lower levels are inadequate or absent. Effective escalation protocols define: when to escalate, to whom, what information to communicate (SBAR), expected response time, and what to do if response is still inadequate.

What is a Structured Communication Tool?

A communication framework that standardizes information exchange to reduce omissions, misunderstandings, and failures during high-stakes clinical interactions. Examples: SBAR (Situation-Background-Assessment-Recommendation), I-PASS (handoff), CUS (safety assertion), Two-Challenge Rule. Structured tools reduce communication-related adverse events.

What is a Simulation in nursing quality?

A technique using realistic patient scenarios with manikins, standardized patients, or virtual platforms to train nurses in clinical skills, emergency response, and team communication without patient risk.

High-fidelity simulation with structured debriefing improves knowledge retention, skill acquisition, and team performance.

Evidence supports simulation for low-frequency, high-stakes competency validation.

What is a Care Coordination Failure?

A breakdown in the communication, information sharing, or handoffs between care team members, settings, or disciplines that results in a gap, duplication, or error in care delivery. Care coordination failures are a leading cause of patient harm at transitions of care. Root causes include: inadequate handoff processes, EHR fragmentation, and unclear role responsibilities.

What is a Hospital Incident Command System (HICS) in quality?

The HICS structure defines accountability and communication during mass casualty events, disasters, and surge situations. Quality implications: surge requires deviation from standard nurse-to-patient ratios, which must be tracked and justified. After the event, a systematic after-action review identifies system performance gaps for quality improvement.

What is a CUSP team?

A unit-based multidisciplinary team that implements the Comprehensive Unit-Based Safety Program. Core activities: educating staff on the science of safety, having each staff member identify one thing that could hurt a patient on their unit today, implementing evidence-based safety tools (e.g., CLABSI bundle), and partnering with a senior executive to address systems issues.

What is Crew Resource Management (CRM) in healthcare?

Adapted from aviation, CRM training teaches healthcare teams effective communication, situational awareness, decision-making, and leadership under pressure. CRM principles: speak up, acknowledge mistakes, cross-check, use checklists, share mental models, distribute workload. TeamSTEPPS is the healthcare adaptation of CRM principles.

What is a Tracer Methodology (Joint Commission)?

An evaluation process in which surveyors follow a patient's care journey through the hospital to evaluate compliance with standards at each point of care. Individual patient tracers are supplemented by system tracers (infection control, medication management, emergency management) that assess organization-wide compliance. It provides a real-world compliance view.

What is a HCAHPS Nurse Communication Composite?

A three-question composite measuring patients' perception of nurse communication: nurses treating with courtesy and respect, nurses listening carefully, and nurses explaining things clearly. It is one of the strongest drivers of overall HCAHPS rating and is directly influenced by nursing practice. Hourly rounding and bedside shift report improve nurse communication scores.

What is a Mock Code in nursing quality?

A simulated cardiac arrest conducted on a nursing unit to test the team's ability to activate the code response, perform CPR, use the defibrillator, manage medications, and communicate with the code team. Mock codes identify equipment gaps, process failures, and competency deficits before they occur in real emergencies. They should be unannounced and debriefed immediately.

What is a Nurse Staffing and Patient Outcomes relationship?

A robust body of research (Aiken, Needleman) shows that adding one patient to a nurse's assignment is associated with a 7% increase in patient mortality (Aiken) and significant increases in HAI rates. Lower nurse-to-patient ratios and higher RN-to-total-nursing-hours ratios are consistently associated with better outcomes on nursing-sensitive indicators.

What is a Hospital Quality Initiative (HQI)?

A coordinated set of evidence-based improvement projects targeting specific quality, safety, or efficiency goals across a hospital or health system. HQIs typically have: executive sponsorship, defined metrics, improvement teams in each target area, a shared measurement platform, and governance-level accountability for results.

What is a Post-Fall Huddle?

A structured team debrief conducted immediately after a patient fall (before end of shift) to: share facts about the fall circumstance, identify contributing factors, assess injury, determine immediate care changes, and document the review. Post-fall huddles are a best practice for accelerating learning and reducing recurrence. They are distinct from the formal incident review.

What is the AHRQ Patient Safety Indicators (PSI-90) composite?

A composite measure of hospital-acquired complications including selected surgical complications, iatrogenic events, and patient safety events. PSI-90 is used by CMS in Hospital-Acquired Condition Reduction Program (HACRP) which penalizes hospitals in the worst-performing quartile by 1% of total Medicare payments.

What is a Rapid Improvement Event (RIE)?

A focused 3-5 day structured workshop using Lean principles to analyze and redesign a specific process. A cross-functional team maps the current state, identifies waste and root causes, designs and implements improvements during the event, and defines a 30-day follow-up monitoring plan. RIEs produce immediate measurable process change.

What is an Incident Command System (ICS) in quality safety?

A standardized hierarchical management structure for responding to emergencies of any scale. ICS ensures clear command, unified communication, modular organization, manageable span of control, and comprehensive resource management. Healthcare organizations use Hospital ICS (HICS) as their emergency management framework, integrated with local and regional emergency systems.

What is a Medication Error near-miss report?

A report describing a medication error that was intercepted before reaching the patient. Near-miss reports are the highest-value safety data because they reveal system vulnerabilities without the distraction of patient harm outcomes. Organizations that encourage near-miss reporting generate more improvement opportunities than those that report only actual adverse events.

What is a High-Reliability Team Behavior?

Behaviors that contribute to HRO team function: briefing before high-risk procedures, speaking up when conditions change, calling out concerns directly and respectfully, escalating without waiting to be asked, cross-checking critical actions, debriefing after events, and valuing learning from every safety signal. HRO team behaviors are trained and practiced, not innate.

What is a Dashboard Indicator Target?

A defined performance threshold for each quality metric that signals whether the organization is meeting expectations. Targets should be: evidence-based (aligned with national benchmarks or best-practice data), achievable but aspirational, reviewed annually, and differentiated from thresholds (the minimum acceptable level below which immediate action is required).

What is a nurse's role in restraint reduction?

Assessing for the least restrictive alternative to physical restraint (de-escalation, sitter, fall mat, enclosure bed), implementing alternatives before initiating restraints, obtaining the required physician order, monitoring restrained patients per policy, reassessing necessity every 2 hours, documenting all assessments, and planning for early discontinuation.

What is a Patient Safety Survey Benchmark?

A comparison score from AHRQ's national HSOPSC database that shows how an organization's patient safety culture composite scores compare to other hospitals. Benchmarks provide context for interpretation: a 60% positive response rate may be high compared to the national average of 52% for the same composite.

What is a Pressure Ulcer Prevention Protocol?

A standardized care protocol triggered by Braden Scale score ≤ 18 that specifies: repositioning frequency (minimum 2-hour schedule), skin inspection documentation, moisture barrier application, support surface upgrade, nutritional consultation, heel protection, and patient/family education. Protocol compliance is tracked and reported as a nursing quality indicator.

What is a TJC Periodic Performance Review (PPR)?

A self-assessment tool provided by TJC to help accredited organizations identify standards compliance gaps between triennial surveys. Organizations use the PPR to evaluate their compliance with the full accreditation manual, prioritize improvement, and demonstrate ongoing compliance commitment. PPR results are reviewed by the accreditation team.

What is a Quality Measure Data Validation?

The process of verifying that quality metric data submitted to regulatory and accreditation bodies accurately represents actual performance. Validation involves: auditing source records against submitted data, checking abstraction accuracy, reconciling denominator definitions, and testing data extraction logic. CMS conducts random validation audits of hospital quality data submissions.

What is a Hospital-Acquired Condition Reduction Program (HACRP)?

A CMS mandatory program that penalizes hospitals in the worst-performing quartile for hospital-acquired conditions by reducing their Medicare payments by 1%. HAC domain measures include: PSI-90 composite (patient safety events) and HAI measures (CLABSI, CAUTI, MRSA, C. diff, SSI). Nurse executives monitor HACRP performance to protect hospital revenue.

What is the Lean concept of standard work?

A precisely documented description of the safest, highest-quality, most efficient way to perform a specific task, based on current best evidence. Standard work specifies: sequence of steps, timing, materials needed, and quality checkpoints. It is the baseline from which improvement is measured. Deviating from standard work is the first source of defects to investigate.

What is a Patient Safety Champion?

A frontline nurse or other clinical staff member who is formally recognized and trained to promote safety culture on their unit. Champions lead safety huddles, role-model reporting behaviors, coach peers on Just Culture principles, and serve as a liaison between unit staff and quality leadership. Champions are essential for sustaining safety improvements between external quality events.

What is a Nursing Audit?

A systematic review of nursing documentation, care delivery, or outcome data against defined standards to evaluate compliance and identify improvement opportunities. Types: concurrent audit (during patient stay), retrospective audit (after discharge), and outcome audit (patient results). Nursing audit findings drive targeted education and protocol updates.

What is an Anticoagulation Safety Program?

A system-level program to manage anticoagulation therapy safely. Components: weight-based dosing protocols, independent double-check for IV heparin, INR monitoring protocols, pharmacist oversight, patient education on warfarin/DOAC safety, fall risk management for anticoagulated patients, and reversal agent availability. Anticoagulants are on the ISMP High-Alert Medication list.

What is a nursing documentation audit result action plan?

A structured response to nursing documentation audit findings that identifies: specific deficiencies and their frequency, root causes (knowledge gap, time constraint, workflow design, EHR usability), targeted interventions (education, workflow redesign, EHR optimization), accountability for implementation, timeline, and re-audit schedule to verify improvement.



Human Capital

Flashcards 426–500

What is Maslow's Hierarchy of Needs applied to nursing retention?

From base to peak: Physiological (safe working conditions, adequate staffing), Safety (secure employment, non-punitive environment), Social (team belonging, collegial relationships), Esteem (recognition, career advancement), Self-Actualization (professional growth, meaningful practice). Retention strategies must address all levels.

What is Herzberg's Two-Factor Theory?

Motivators (intrinsic): achievement, recognition, work itself, responsibility, advancement — their presence creates satisfaction. Hygiene Factors (extrinsic): salary, working conditions, supervision, company policy — their absence causes dissatisfaction but their presence does not create satisfaction. Nursing retention requires addressing both.

What is Nurse Turnover Cost?

The total financial cost of replacing one nurse, including: separation costs, vacancy costs, recruitment costs, onboarding costs, orientation costs, and productivity loss. Estimates range from 75-200% of annual salary or \$82,000-\$100,000 per nurse. Reducing turnover by even 5% can save a hospital millions annually.

What is the Nurse Retention Toolkit (ANA/Healthy Nurse)?

Evidence-based strategies for retaining nurses: competitive compensation, flexible scheduling, shared governance, professional development pathways, career ladders, recognition programs, new graduate residency programs, manager development, safe staffing, and workplace safety. No single strategy is sufficient — bundled approaches work best.

What is a Nurse Residency Program?

A structured one-year post-graduation program that supports new nurses through the transition from student to professional practice. Evidence shows nurse residency programs reduce first-year turnover from 20-30% to 10-15% and improve clinical competency, confidence, and patient outcomes. CCNE accreditation validates program quality.

What is Benner's Novice-to-Expert Model?

Patricia Benner's five-stage model of nursing expertise: Novice (rule-based, no situational experience), Advanced Beginner (guidelines, some experience), Competent (organized, deliberate planning), Proficient (situational perception, holistic view), Expert (intuitive, fluid performance). Development requires experience in context, not just training.

What is a Clinical Ladder Program?

A formal career advancement pathway that recognizes and rewards clinical excellence and professional growth without requiring nurses to move into management. Advancement criteria typically include clinical competency, professional development, evidence-based practice contribution, leadership activity, and patient outcomes. Ladders improve retention and professional development.

What is the ADA (Americans with Disabilities Act) in nursing employment?

The ADA prohibits discrimination against qualified individuals with disabilities in all aspects of employment. Employers must provide reasonable accommodation unless it causes undue hardship. The interactive process must be engaged whenever a disability accommodation is requested. Healthcare workers with HIV, psychiatric disorders, and chronic illness may qualify.

What is the FMLA (Family and Medical Leave Act)?

Federal law requiring employers with 50+ employees to provide up to 12 weeks of unpaid, job-protected leave per year for qualifying medical or family reasons. FMLA restoration rights require return to the same or equivalent position with the same terms and conditions, including schedule, shift, and workplace accommodations.

What is the FLSA (Fair Labor Standards Act)?

Federal law establishing minimum wage, overtime pay (1.5× regular rate for hours over 40/week for non-exempt employees), child labor standards, and recordkeeping requirements. RNs in supervisory roles may be exempt. Non-exempt nursing staff (e.g., nursing assistants) must receive overtime pay for hours over 40 per week.

What is Title VII of the Civil Rights Act?

Federal law prohibiting employment discrimination based on race, color, religion, sex, or national origin. The 4/5ths (80%) rule is used for adverse impact analysis: if the selection rate for a protected group is less than 80% of the selection rate for the highest-selected group, adverse impact is presumed. Applies to hiring, promotion, and termination.

What is the Pregnancy Discrimination Act (PDA)?

An amendment to Title VII prohibiting discrimination based on pregnancy, childbirth, or related medical conditions. Employers must treat pregnancy-related conditions the same as any other temporary medical condition. Nurse executives must ensure that pregnant nurses are offered the same accommodations available to other employees with medical conditions.

What is the ADEA (Age Discrimination in Employment Act)?

Federal law prohibiting employment discrimination against persons 40 years of age or older. Applies to hiring, firing, promotion, pay, and terms of employment. Performance improvement plans or termination decisions that affect primarily older workers must be based on documented, objective performance criteria to avoid ADEA claims.

What is the NLRA Section 7 and protected concerted activity?

Section 7 of the National Labor Relations Act protects employees' rights to discuss wages, hours, and working conditions with coworkers and to engage in concerted activity to improve those conditions. This right applies to both union and non-union employees. Disciplining employees for salary discussions or collective safety complaints violates the NLRA.

What is Collective Bargaining in nursing?

A formal process in which union representatives negotiate with management on behalf of union members to establish or modify terms of employment, including wages, hours, scheduling, workload, and working conditions. Nurse executives must understand CBA language, adhere to grievance procedures, and maintain good-faith bargaining obligations.

What is a Grievance Process in unionized hospitals?

A formal multi-step dispute resolution process specified in the collective bargaining agreement (CBA) for resolving alleged violations of contract terms. Steps typically progress from informal resolution to formal investigation to arbitration. Nurse executives must respond within CBA-defined timeframes and document all steps.

What is the Kirkpatrick Model of Training Evaluation?

A four-level framework: Level 1 (Reaction — did participants like the training?), Level 2 (Learning — did knowledge/skills increase?), Level 3 (Behavior — did behavior change back on the unit?), Level 4 (Results — did patient outcomes improve?). Most nursing training evaluations stop at Level 1 or 2; Levels 3 and 4 provide more meaningful ROI evidence.

What is Competency-Based Education in nursing?

An educational approach in which learners demonstrate defined knowledge, skills, and behaviors to a specified performance standard, regardless of time in training. Nurse executives use competency frameworks to: design orientation, validate return-to-work readiness, assess cross-training requirements, and define clinical ladder criteria.

What is a Staff Competency Assessment?

A systematic process for evaluating whether nurses demonstrate the knowledge, skills, and behaviors required for safe practice in their specific role and unit. Annual competency assessment should include high-risk, low-frequency skills, equipment changes, new procedures, and clinical updates — not routine skills already demonstrated daily.

What is a Preceptor Program?

A structured clinical learning program pairing new nurses with experienced preceptors for a defined orientation period. Effective preceptor programs include: preceptor selection criteria (clinical competency + teaching aptitude), preceptor training, structured competency checklists, regular feedback meetings, and preceptor recognition and compensation.

What is Succession Planning for nurse managers?

A deliberate process for identifying, developing, and preparing nurses to advance into manager or director roles. Components: talent identification, formal leadership development programming, role-shadowing, stretch assignments, mentoring, and a defined advancement timeline. Succession planning must address equity in advancement opportunity.

What is an Employee Assistance Program (EAP)?

A confidential employer-sponsored program providing short-term counseling, referral, and follow-up services for employees with personal, family, mental health, substance use, or work performance challenges. EAP is a non-disciplinary resource. Nurse executives should actively promote EAP access and reduce stigma around mental health support.

What is a Nurse Manager Burnout Prevention Strategy?

Evidence-based strategies include: right-sizing spans of control (≤ 12 direct reports), providing leadership development and coaching, structuring administrative support to reduce non-clinical burden, creating peer learning communities, recognizing manager contributions, and modeling healthy work-life boundaries from senior leadership.

What is Moral Distress in nursing?

Suffering experienced when nurses know the ethically right action but are constrained from taking it by institutional, authority, or role barriers. Moral distress is a driver of burnout, compassion fatigue, and turnover. Nurse executives must create mechanisms for nurses to raise ethical concerns and must address systemic sources of moral distress.

What is Compassion Fatigue?

A state of physical, emotional, and spiritual exhaustion resulting from caring for those in pain or suffering. It differs from burnout in its trauma exposure component. Signs include reduced empathy, emotional numbness, cynicism, and difficulty concentrating. Prevention includes peer support, workload management, and debriefing after difficult cases.

What is the Professional Quality of Life Scale (ProQOL)?

A validated tool measuring three constructs: Compassion Satisfaction (positive feelings from helping), Burnout (hopelessness and difficulties), and Secondary Traumatic Stress (traumatic stress related to patient exposure). Widely used in nursing to assess workforce well-being and target well-being program interventions.

What is a Staffing Plan?

A formal document specifying how a nursing unit will be staffed across all shifts, census levels, and acuity ranges. It includes: FTE budget, scheduling parameters, on-call requirements, float policies, minimum staffing thresholds, overtime management, and escalation processes for staffing below minimum. TJC requires written staffing plans.

What is Mandatory Overtime in nursing?

The practice of requiring nurses to work beyond their scheduled shift without a prior voluntary agreement. ANA opposes mandatory overtime as a staffing solution. Evidence links mandatory overtime to increased medication errors, patient falls, and nurse fatigue injuries. Some states have enacted legislation restricting mandatory overtime for nurses.

What is a Nurse Staffing Agency (Travel Nurse)?

A vendor that places temporary nurses (typically on 13-week contracts) to fill vacancies at client hospitals. Travel nurses typically earn 50-100% more per hour than permanent staff, creating pay equity tension. High travel nurse dependency (>15% of shifts) indicates a structural vacancy problem requiring systemic workforce intervention.

What is Adverse Impact Analysis (EEOC 4/5ths Rule)?

A method for detecting employment discrimination. If the selection rate for a protected group is less than 80% of the selection rate for the highest-selected group in any employment decision (hiring, promotion, training), adverse impact is presumed and requires investigation or adjustment. Applies to all employment decisions, not just hiring.

What is a Job Analysis?

A systematic process for identifying the knowledge, skills, abilities, and other characteristics (KSAOs) required for a specific job. Job analyses are the foundation of valid job descriptions, competency frameworks, interview questions, and performance evaluation criteria. They protect against discrimination claims when used consistently.

What is a Performance Management Cycle?

A continuous cycle: Plan (set goals and competency standards), Monitor (track progress with ongoing feedback), Develop (coaching, training, stretch assignments), Rate (formal annual or semi-annual evaluation), Reward (recognition, advancement, compensation). The cycle must be documented and applied consistently.

What is a Performance Improvement Plan (PIP) legal requirements?

A PIP must include: specific, objective, and measurable performance expectations; a realistic timeline; support the organization will provide; consequences for non-achievement; and consistent application across similarly situated employees. PIPs that are subjectively applied, impossible to achieve, or applied selectively create discrimination liability.

What is a Workforce Diversity Strategy?

A systematic organizational approach to building and retaining a diverse nursing workforce. Components: pipeline partnerships with community colleges, equitable hiring practices, anti-bias interview training, mentoring programs for underrepresented nurses, role-stratified representation analysis, and equity audits of promotion and compensation decisions.

What is a Safe Harbor Provision in nursing?

A formal policy allowing a nurse to document objection to an unsafe assignment without immediate disciplinary action, pending a clinical review. Texas and other states have enacted safe harbor provisions in their Nurse Practice Acts. Without formal safe harbor, nurses who refuse unsafe assignments risk disciplinary action despite safety justification.

What is the Role of the Charge Nurse in staffing?

The charge nurse manages shift-level patient assignments, adjusts staffing for acuity changes, requests additional resources from the house supervisor, and ensures patient flow. The charge nurse is distinct from the unit manager and from the nursing supervisor. Role clarity and formal orientation for charge nurses are essential for patient safety.

What is a Nurse Peer Review?

A formal process in which a nurse's clinical practice is evaluated by a group of peers using evidence-based criteria. Peer review is part of professional accountability and is required by many state boards of nursing and TJC. It differs from disciplinary review: peer review is forward-looking and developmental, focused on improving practice.

What is an Exit Interview?

A structured conversation or survey conducted with employees upon voluntary separation to identify reasons for departure. Exit interview data analyzed in aggregate reveals patterns (manager quality, scheduling, compensation, workload) that can guide retention strategy. Individual exit interview comments should be treated confidentially.

What is a Nurse Manager Competency Framework?

A defined set of knowledge, skills, and behaviors required for effective nurse manager performance. AONE (American Organization of Nurse Executives) Nurse Manager Competencies cover five domains: personal and professional accountability, career planning, performance management, relationship management and influence, and diversity. Used for selection, development, and evaluation.

What is Workplace Accommodation Process (ADA)?

The interactive process required when an employee requests accommodation for a disability. Steps: receive and document the request, schedule a meeting with the employee, review medical documentation from the treating provider, identify reasonable accommodation options, evaluate undue hardship, and document the decision and rationale. All steps must be completed before denial.

What is a Market Compensation Analysis?

A systematic comparison of an organization's nurse compensation levels to external market data to determine competitive positioning. Sources include: BLS Occupational Employment Statistics, regional salary surveys, and hospital peer group comparisons. Compensation analysis should be conducted annually and must be stratified by experience level, specialty, and geography.

What is a Nurse Manager Span of Control?

The number of direct reports a nurse manager oversees. Evidence shows effective nurse manager spans for complex acute care units are 8-12 direct care staff. Spans above 12 reduce the manager's ability to provide developmental oversight, conduct timely performance evaluations, and maintain situational awareness of unit quality and culture.

What is an Employee Engagement Survey?

A structured tool measuring how committed, motivated, and connected employees feel to their work and organization. Key dimensions: pride, advocacy, commitment, discretionary effort. Press Ganey, Gallup, and Studer Group are common vendors. Response rates matter: a 22% response rate cannot be generalized to represent the workforce.

What is a Just Culture policy for nursing staff?

A formal policy defining how the organization responds to staff involved in safety events. It specifies: event classification criteria (human error vs. at-risk vs. reckless), corresponding responses (console vs. coach vs. discipline), the investigation process, and protection against retaliation for voluntary self-reporting. Just Culture policy must be consistently applied across all roles.

What is a Float Pool Nurse Competency?

The defined skills, knowledge, and validated competencies required for nurses assigned to float across multiple units. Float pool nurses must demonstrate unit-specific competencies for each area they rotate to, including high-alert medication differences, equipment, patient population characteristics, and emergency response procedures.

What is Succession Pipeline vs. Individual Succession Planning?

Pipeline planning: building a general pool of future leaders through broad development programs. Individual succession planning: specifically preparing named individuals for specific roles. Healthcare organizations need both. Pipeline planning develops bench depth; individual succession ensures critical roles have ready replacements for planned and unplanned vacancies.

What is a Labor Relations Specialist role?

An HR professional or attorney who advises management on union contract compliance, grievance management, bargaining strategy, and NLRA obligations. Nurse executives in unionized hospitals should partner with labor relations specialists for all matters involving the collective bargaining agreement and for any communication directly with union representatives.

What is a Retention Bonus?

A financial incentive paid to employees who remain employed for a specified time period (typically 1-2 years). Evidence shows retention bonuses effectively reduce short-term turnover but do not address underlying dissatisfiers. Nurses who accept bonuses despite intending to leave generate a post-bond departure spike. Bonuses alone are not a retention strategy.

What is a Nurse Workforce Diversity Pipeline?

A systematic approach to increasing diversity in the nursing workforce by removing structural barriers at every stage: recruitment from diverse communities, academic support programs for underrepresented nursing students (stipends, tutoring), clinical placement in the hiring hospital, and employment pathways upon graduation.

What is a Peer Support Program for nurses?

A structured program providing emotional support from trained nurse peers to colleagues who have experienced a traumatic patient care event (e.g., unexpected death, adverse event, code). Peer supporters are trained in supportive listening, resource referral, and psychological first aid. Evidence shows peer support programs reduce burnout and post-traumatic distress.

What is HPPD calculation for staffing?

Hours Per Patient Day = Total nursing hours worked ÷ Patient days. Example: 8,760 patient days × 8.2 HPPD target = 71,832 total hours needed. Divide by 2,080 (hours per FTE per year) = 34.5 direct care FTEs needed. HPPD targets are set by unit type, acuity level, and regulatory requirements.

What is a nurse's legal scope of practice?

The legally defined boundaries of what a licensed nurse may do, as specified by the state's Nurse Practice Act and implementing rules. Scope of practice is different from job description (which specifies what the employer assigns) and competency (what the nurse has demonstrated). Practicing outside scope of practice creates personal legal liability and grounds for license revocation.

What is a nursing job satisfaction survey?

A tool measuring nurses' perceptions of their work environment, including workload, autonomy, relationships, recognition, advancement, and management support. Press Ganey Nurse Engagement, the Practice Environment Scale-NWI (PES-NWI), and the RN4CAST survey are widely used. Results are benchmarked and used to prioritize retention improvement initiatives.

What is the Practice Environment Scale-Nursing Work Index (PES-NWI)?

A validated 31-item survey measuring five dimensions of the nursing work environment: Nurse Participation in Hospital Affairs, Nursing Foundations for Quality of Care, Nurse Manager Ability, Leadership and Support, Staffing and Resource Adequacy, and Collegial Nurse-Physician Relations. Magnet hospitals consistently score higher than non-Magnet hospitals on PES-NWI subscales.

What is progressive discipline?

A structured approach to addressing performance or conduct problems that escalates the severity of consequences if behavior does not improve. Steps: verbal counseling, written warning, final written warning (with PIP), suspension without pay, termination. Each step is documented. Progressive discipline must be applied consistently to avoid discrimination claims.

What is a Job Rotation program in nursing?

A workforce development strategy that provides nurses with planned assignments across different units, roles, or settings to broaden clinical skills, reduce monotony, build organizational understanding, and prepare staff for advancement. Job rotation is used in leadership development programs, cross-training initiatives, and residency programs.

What is the ANCC Magnet® designation impact on nursing workforce?

Magnet-designated hospitals consistently report: lower nurse turnover, higher nurse satisfaction, lower HPPD-adjusted nursing costs, better nurse-sensitive patient outcomes, and better nurse recruitment outcomes (particularly for specialty-certified nurses). The Magnet journey itself — even before designation — improves nursing practice environments.

What is a Nurse Externship Program?

A structured clinical work experience for nursing students (typically between their junior and senior year) providing supervised clinical exposure, professional development, and an employment pipeline. Nurse externs often convert to new graduate hires, reducing recruitment cost and accelerating competency development. Externs require structured supervision by experienced RNs.

What is a Compensation Philosophy?

An organization's formal position on how it will set and manage employee compensation relative to the market, including: target market position (e.g., 50th percentile for base pay), pay-for-performance approach, equity commitments, and total rewards philosophy.

Nurse executives should ensure the organization's compensation philosophy is competitive enough to support nursing recruitment and retention.

What is a nursing certification incentive program?

An organizational program that encourages and rewards nurses for obtaining and maintaining specialty nursing certifications. Common incentives: one-time bonus, hourly premium, exam fee reimbursement, study resource support, and recognition programs. Magnet designation requires documenting nursing certification rates as a structural empowerment measure.

What is an accommodation under the ADA for a nurse?

A modification to the nurse's job duties, schedule, equipment, or work environment that allows them to perform the essential functions of their position despite a qualifying disability. Examples: modified lifting restrictions, altered work schedule, ergonomic equipment, or modified job duties. Accommodations must not fundamentally alter the essential job functions or impose undue hardship.

What is the Fair Labor Standards Act (FLSA) overtime threshold?

FLSA requires non-exempt employees to receive overtime pay at 1.5× their regular rate for all hours worked over 40 in a workweek. Salaried employees qualify as exempt only if: they meet the salary threshold (currently \$684/week minimum) AND perform executive, professional, or administrative duties as defined by FLSA. Misclassifying nurses as exempt creates significant wage liability.

What is nursing staff scheduling best practice?

Best practices: self-scheduling within defined parameters (shift requirements, skill mix minimums), 4-6 weeks advance posting, equitable distribution of weekends/holidays, maximum 12-hour shifts with adequate rest between shifts, limits on consecutive shifts, and a transparent process for resolving conflicts. Self-scheduling improves nurse satisfaction without sacrificing coverage requirements.

What is a 4/5ths (80%) rule in employment testing?

EEOC adverse impact analysis guideline: if any protected group's selection rate (hired, promoted, passed) is less than 80% of the group with the highest rate, adverse impact is indicated. Example: if 50% of white applicants are hired but only 30% of Black applicants are hired, $30/50 = 60\%$, which is below 80%, indicating adverse impact requiring justification.

What is a Return-to-Work Program in nursing?

A structured process for returning injured or ill nurses to work, often through modified duty assignments that meet medical restrictions while keeping the nurse employed. Effective RTW programs reduce workers' compensation costs, prevent prolonged disability, support nurse retention, and maintain the therapeutic benefit of productive work during recovery.

What is Occupational Health nursing?

A specialty nursing practice focused on protecting and promoting the health, safety, and productivity of workers. In hospitals, occupational health nurses manage work-related injuries and illnesses, TB testing, immunization programs, bloodborne pathogen exposure follow-up, fit-for-duty evaluations, and RTW programs for nursing staff.

What is a union organizing campaign?

A process in which employees (or an external union) seek to gain collective bargaining rights at a workplace. Once 30% of eligible employees sign authorization cards, the NLRB can hold an election. During organizing, management cannot: threaten, interrogate, make promises, or spy on organizing activity (TIPS rules). Violations are Unfair Labor Practices under the NLRA.

What is the difference between RN and LPN scope of practice?

RNs perform comprehensive nursing assessment, formulate nursing diagnoses, develop nursing care plans, initiate interventions, administer medications, and supervise LPNs and UAPs. LPNs/LVNs perform directed nursing care within a defined, more limited scope, contributing to data collection and implementing assigned interventions under RN supervision. Specific scope differences vary by state Nurse Practice Act.

What is a Generation-Specific Retention Strategy?

Retention strategies tailored to generational workforce characteristics. Baby Boomers value stability, mentoring, and contributions being recognized. Gen X values autonomy, work-life balance, and clear feedback. Millennials value purpose, collaboration, technology, and advancement. Gen Z values flexibility, social impact, inclusion, and mental health support. Effective nurse executives deploy multi-generational approaches.

What is a Clinical Nurse Leader (CNL) role?

A master's-prepared generalist nurse who leads care coordination and quality improvement at the microsystem (unit) level. CNLs are not advanced practice nurses or managers — they are lateral integrators who assess care outcomes, identify unit-level improvement opportunities, implement evidence-based practices, and educate staff. CNLs are an AACN initiative.

What is the ANCC NE-BC certification eligibility requirement?

To sit for the NE-BC exam, nurses must: hold a current RN license, have a baccalaureate or higher degree in nursing, and have a minimum of 24 months of employment as a nurse administrator/executive in the previous 5 years, with at least 30% of time in administrative responsibilities. The exam tests competency across four domains of nurse executive practice.

What is an annual performance evaluation in nursing?

A formal documented assessment of a nurse's performance against defined job expectations and competencies conducted at least annually. Effective evaluations include: self-assessment, manager assessment, objective performance data, specific examples of behavior, developmental goals, and a two-way conversation. Evaluations must be completed on time and stored in the personnel file.

What is an Integrated Workforce Plan?

A comprehensive strategic plan aligning nursing workforce supply (recruitment, retention, development) with projected demand (patient volume, new services, retirements, turnover). It includes: current workforce gap analysis, 3-5 year staffing projections, hiring targets by role, education pipeline partnerships, succession plan, and financial modeling for workforce investment.

What is a nurse executive's legal risk in staffing decisions?

Nurse executives face legal risk when staffing decisions contribute to patient harm (respondeat superior/corporate negligence), when staffing violates minimum ratio laws (civil and criminal penalties), when staffing decisions discriminate against protected classes, or when nurses are retaliated against for raising safe staffing concerns. Documented staffing decision-making processes provide evidence of reasonable care.

What is a workplace wellness program for nurses?

A comprehensive program supporting nurses' physical, mental, and emotional well-being to prevent burnout, reduce absenteeism, and sustain a healthy workforce. Evidence-based components: mindfulness programs, ergonomics and safe patient handling training, fitness resources, healthy food options, peer support programs, mental health access, and scheduling practices that allow adequate rest between shifts.